

ACCESS / OPERATIONS / PRIVACY / PATIENT COMMUNICATION

Healthcare Administration English

Clarify the status. Coordinate the next step. Keep the patient informed.

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Eight lessons. Eleven extended conversations.

INSIDE THE BOOK

192 vocabulary entries / 128 reusable phrases
220 extended-dialogue turns / 11 transfer exchanges
Word banks and explained answer keys

Upper-intermediate to advanced (B2-C1). For patient-access, operations, revenue-cycle, quality, coordination, and administrative leadership teams.

2026 edition

Eight scenarios. Eight skills.

Practice eight administrative conversations in which precise status language, ownership, and follow-through matter.

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A clear handoff includes an owner.

Healthcare administration connects people, records, schedules, and decisions. A referral received is not an appointment booked; a discharge planned is not a bed available; a message sent is not a handoff accepted. These fictional cases practice the language needed to preserve those distinctions while remaining considerate and useful.

Tell the patient what is known

Acknowledge the request, explain the missing administrative step in ordinary language, and name the next update. Avoid making patients decode internal queue names or promises that depend on an unconfirmed clinical decision.

"We received the referral. I am checking the missing information and will update you by three."

Separate current status from expected status

Forecasts help teams plan, but they must remain visibly conditional. Confirm what has actually happened before reporting capacity, payment, transport, or approval as available.

"Two discharges are planned; neither bed is confirmed available now."

Match access to the actual task

A colleague's job title alone does not settle the scope of a data request. Confirm purpose, role, applicable permissions, and the appropriate sharing route through organizational policy.

"Which fields are needed for this task, and has the requested access been confirmed?"

Close the communication loop

A named owner, a next action, an update time, and acknowledgment make a handoff usable. Administrative follow-up does not replace clinical judgment or the appropriate route for urgent concerns.

"I will confirm receipt and record who owns the unresolved transport request."

PROFESSIONAL BOUNDARIES

Original fictional language practice, not medical, legal, billing, coding, or compliance advice. US terminology appears where identified. Follow applicable law, payer rules, privacy safeguards, current organizational procedures, and qualified clinical judgment. No real patient information is used.

The referral arrived, but the booking is not ready

A Read the situation

Request missing referral information and give a patient a reliable update without inventing clinical details.

Scheduler Elena receives a referral with patient identifiers and contact details but no requested specialty or reason for the appointment. The patient expects a booking confirmation today. No appointment has been reserved and no clinical priority has been assigned. Referral-office colleague Marcus can seek clarification from the referring team. Elena can promise a status update by 3 p.m., not a confirmed appointment. Clinical urgency questions must follow the clinical review process rather than a scheduler's guess.

PEOPLE / Elena: Scheduler | Marcus: Referral-office colleague

MAKE THE NEXT STEP CONCRETE

Patients may hear received as ready to book. Explain the difference without blaming the referring office or listing internal obstacles. Promise an update you can control, identify who is seeking the missing information, and keep clinical prioritization with the appropriately qualified team.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What information is missing?

- | | |
|---|---|
| A Every patient identifier and contact detail | B A confirmed appointment that already exists |
| C A documented clinical priority marked urgent | D The requested specialty and reason for the appointment |

2. What can Elena reliably promise on the supplied facts?

- | | |
|--|---|
| A A particular specialist's clinical assessment | B A same-day consultation regardless of availability |
| C A status update by 3 p.m. | D A confirmed appointment by 3 p.m. |

3. Who should determine clinical urgency?

- | | |
|---|--|
| A The appropriately qualified clinical team through its review process | B The scheduler from an assumption about the referral |
| C The patient-access dashboard from receipt time alone | D The newest staff member to open the file |

[Answers and explanations: page 85.](#)

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

patient access

Administrative processes helping patients reach appropriate services.

IN USE coordinate patient access

referral

A request directing a patient to another service or professional.

IN USE process a referral

referring clinician

The clinician requesting another service or consultation.

IN USE contact the referring clinician

receiving service

The team or department expected to act on a referral.

IN USE identify the receiving service

specialty

A defined branch of clinical practice.

IN USE confirm the requested specialty

reason for referral

The stated clinical purpose of the requested consultation.

IN USE clarify the reason for referral

referral completeness

Whether the required referral information has been supplied.

IN USE check referral completeness

clinical triage

Qualified assessment of clinical priority and appropriate next care steps.

IN USE route for clinical triage

appointment slot

A defined time available for a scheduled visit.

IN USE confirm an appointment slot

scheduling template

The configured pattern of appointment types and times.

IN USE review the scheduling template

booking confirmation

A message establishing that an appointment has been arranged.

IN USE issue a booking confirmation

provisional hold

A temporary reservation subject to stated conditions.

IN USE distinguish a provisional hold from a booking

Precision check

Received means the referral arrived; complete means the required information is present; booked means an appointment is arranged. None of these statuses automatically establishes clinical urgency, insurance coverage, or authorization. State each confirmed fact separately.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

waitlist

A managed list of patients awaiting an available appointment.

IN USE maintain the waitlist

cancellation list

A list used to offer newly available appointments.

IN USE check the cancellation list

intake queue

The worklist holding incoming requests for initial processing.

IN USE monitor the intake queue

demographic verification

Checking patient identity and contact details for accuracy.

IN USE complete demographic verification

eligibility verification

Checking current insurance or service eligibility information.

IN USE request eligibility verification

network status

Whether a provider participates in the relevant insurance network.

IN USE confirm network status

authorization requirement

A condition requiring the relevant approval before a specified service or coverage.

IN USE check authorization requirements

interpreter request

A request for appropriate language assistance.

IN USE arrange an interpreter request

accessibility need

A requirement affecting a person's ability to use a service.

IN USE record an accessibility need

callback commitment

An explicit promise to contact someone again by a stated time.

IN USE honor a callback commitment

closed-loop referral

A referral process with tracked receipt, action, and feedback to the relevant parties.

IN USE support closed-loop referrals

escalation pathway

The designated route for unresolved or urgent issues.

IN USE follow the escalation pathway

Precision check

A callback commitment promises communication, not a clinical decision or available appointment. Record the owner and time. If clinical urgency arises, follow the appropriate clinical escalation process; an administrative update deadline must not become a reason to delay necessary clinical attention.

Phrases you can actually use

C Notice the language / 1 of 2

ACKNOWLEDGE RECEIPT

"We received your referral and are checking the information needed to book."

REQUEST CLARIFICATION

"Please confirm the intended receiving service with the referring team."

SET AN UPDATE

"I will contact you by 3 p.m. with the current status."

MARK BOOKING STATUS

"No appointment has been reserved yet."

NAME THE MISSING DETAIL

"The requested specialty and appointment reason are not shown."

AVOID AN INVENTED REASON

"I cannot supply a clinical reason from the contact details alone."

SEPARATE THE PROMISE

"That is an update time, not a confirmed appointment time."

ROUTE URGENCY

"Clinical priority needs review by the qualified clinical team."

Received versus accepted

Receipt alone may not establish that the receiving service can act.

By three

Specify whether this means an update or a completed booking.

Urgent

Do not infer a clinical category from impatience, a missing field, or an administrative deadline.

Phrases you can actually use

C Notice the language / 2 of 2

TAKE OWNERSHIP

"I will track the clarification request rather than ask you to restart it."

SUPPORT ACCESS

"Let us confirm any interpreter or accessibility arrangements through the usual process."

CHECK RECEIPT

"Please confirm that the referring team received the clarification request."

AVOID A GUARANTEE

"I cannot promise an available slot before the receiving service is confirmed."

CONFIRM THE CONTACT

"What is the appropriate contact route recorded for this request?"

KEEP THE PATIENT INFORMED

"We are waiting for specific information, and I will update you even if it is still pending."

PRESERVE THE DISTINCTION

"A provisional hold would still need its conditions explained."

CLOSE THE LOOP

"Record the response, next action, owner, and patient update."

Clarify rather than diagnose

Ask for the clinical reason; do not invent it.

Ownership

A transfer to another queue should not erase responsibility for the promised update.

Coverage

Verification and authorization have distinct meanings and do not automatically guarantee payment.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which message matches the referral's actual status?

- A** We received the referral and need its specialty and reason; I will update you by 3 p.m.
- B** Your referral is accepted, with the appointment awaiting only a convenient time.
- C** Your booking is provisional because the patient identifiers have been checked.
- D** The referral is routine, and three o'clock is the consultation window.

2. Which clarification should Marcus request?

- A** A clinical reason reconstructed from the patient's address
- B** Permission to call the referral complete without further information
- C** The intended specialty and clinical reason from the referring team
- D** A specialty selected to match today's first free slot

3. What does the absence of an assigned priority mean?

- A** Clinical priority is unconfirmed.
- B** The patient is definitely low priority.
- C** The patient automatically requires the highest priority.
- D** Receipt time alone establishes the clinical category.

4. At 3 p.m., clarification is still pending. What honors the commitment?

- A** Wait silently until an appointment can be guaranteed.
- B** Report a booking that has not been made.
- C** Close the request because the deadline passed.
- D** Contact the patient with the current status and a clear next update.

[Answers and explanations: page 85.](#)

The referral arrived, but the booking is not ready

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

callback commitment / interpreter request / escalation pathway / referring clinician / clinical triage / closed-loop referral / scheduling template / booking confirmation / intake queue / referral completeness

- 01 **Elena:** The referral arrived this morning, but the specialty and appointment reason are blank. The patient expects a booking confirmation today. I need to explain what happens next.
- 02 **Marcus:** Start with ¹ _____. We have identifiers and contact details, but not enough information to identify the requested service. Receipt and readiness to book are different statuses.
- 03 **Elena:** The patient may think we lost the referral if I say we cannot confirm a visit yet. I want to acknowledge that it is here.
- 04 **Marcus:** Say it is in the ² _____ and explain the missing details in ordinary language. Avoid giving the patient an internal queue name as though that answers the practical question.
- 05 **Elena:** Could you ask the referring office to confirm the specialty? I should not choose whichever service happens to have an opening this afternoon.
- 06 **Marcus:** I will contact the ³ _____ through the appropriate office route. The request will identify both missing fields so we do not resolve one and overlook the other.
- 07 **Elena:** We also have no clinical priority recorded. I cannot assume that the missing reason means the request is routine or unimportant.
- 08 **Marcus:** Correct. Any urgency question belongs in ⁴ _____. Follow the established clinical review pathway instead of assigning a category from incomplete administrative information.
- 09 **Elena:** No appointment is reserved. I can update the patient by three, but I cannot promise that the missing information or a suitable slot will be available then.
- 10 **Marcus:** Make that a ⁵ _____. State the time and your responsibility clearly. The patient should hear a reliable next contact, not an implied promise of a consultation.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 10.

- 11 **Elena:** I will say we received the referral and are checking two details before booking. I can avoid blaming the other office or making the patient repeat everything.
- 12 **Marcus:** That distinguishes the update from a ⁶ _____. If the information is still pending at three, contact the patient anyway and explain the next step rather than letting the promise expire silently.
- 13 **Elena:** Once the specialty is confirmed, we still need the receiving team to identify the appropriate appointment type. A general opening may not fit the requested service.
- 14 **Marcus:** Yes. The ⁷ _____ needs to match the confirmed request. We should not turn an available time into an unsuitable booking simply to make the status look complete.
- 15 **Elena:** If the patient needs language assistance or an access arrangement, that should be recorded through our usual process before the visit is finalized.
- 16 **Marcus:** Include an ⁸ _____ where appropriate, and check other relevant access needs. The booking should be usable for the patient, not merely occupy a line on the calendar.
- 17 **Elena:** I will update the patient by three. Please confirm when the referring office receives your request, and tell me if you need help reaching them.
- 18 **Marcus:** That gives us a ⁹ _____ process rather than two disconnected messages. We need a visible response and next action, not just evidence that someone pressed send.
- 19 **Elena:** If the usual contact cannot resolve it, who takes the next step? I want a clear route while I keep the patient updated.
- 20 **Marcus:** Follow the ¹⁰ _____ and preserve the distinction between administrative follow-up and clinical judgment. The immediate commitment remains a clear update by three, with no invented appointment or priority.

ANSWERS / EXPLANATIONS: PAGE 86

1. referral completeness | 2. intake queue | 3. referring clinician | 4. clinical triage | 5. callback commitment | 6. booking confirmation | 7. scheduling template | 8. interpreter request | 9. closed-loop referral | 10. escalation pathway

A callback is not a consultation

F Say it

A scheduler promises a status call by 2 p.m. The specialist has not reviewed the referral, and no appointment is reserved. At 2 p.m., review is still pending.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

pending / reserved / confirmation / call

Scheduler: "I promised a status ¹ _____."

Colleague: "The specialist review remains ² _____."

Scheduler: "No appointment is ³ _____."

Colleague: "Make the promised update without implying a booking
⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 86

1. call | 2. pending | 3. reserved | 4. confirmation

Rehearse with a fixed script

1. Check the cloze key, then read the referral exchange. Keep identifiers received separate from the missing specialty and appointment reason.
2. Switch roles for the scheduling discussion. Stress update by three, no reserved slot, and clinical priority unconfirmed; do not choose a specialty for convenience.
3. Complete and check the two-o'clock callback exchange. Make the promised status call while review remains pending, without calling it a consultation.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

One exception total hides two kinds of follow-up

A Read the situation

Distinguish submission rejections from adjudicated denials and assign follow-up using the actual response record.

A dashboard lists 20 exceptions among 100 submitted claims. The underlying records show 12 rejected before adjudication because required member identifiers were missing or invalid, and eight denied after payer review. The other 80 claims have not been analyzed in this meeting and must not be called paid. Revenue-cycle manager Leo and billing analyst Tessa need separate worklists, reasons, owners, and applicable deadlines. They must verify payer-specific correction or appeal routes, not change clinical codes merely to obtain payment.

PEOPLE / Leo: Revenue-cycle manager | Tessa: Billing analyst

NAME THE STAGE BEFORE NAMING THE SOLUTION

A rejected submission and a denied claim may require different follow-up. Start with the actual acknowledgment or remittance record, not a dashboard label. Explain the stage, reason, and next responsible team without promising payment or treating every unpaid item as a patient balance.

Briefing checks

Circle one answer in each item. Use the briefing.

1. How many exceptions were rejected before adjudication?

- | | |
|-----------------|-----------------|
| A Twenty | B Eighty |
| C Twelve | D Eight |

2. What is known about the other 80 claims?

- | | |
|---|---|
| A They were not analyzed in this meeting. | B All were paid in full. |
| C All were denied for missing identifiers. | D They are all patient balances. |

3. What must guide the correction or appeal route?

- | | |
|---|---|
| A The actual response and applicable payer rules | B The assumption that every exception uses the same appeal |
| C A change to whichever diagnosis code pays more | D The color of the dashboard category |

Answers and explanations: page 87.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

revenue cycle

The sequence of administrative and financial processes from patient access through payment follow-up.

IN USE manage revenue-cycle performance

claim submission

Sending a request for payment to a payer.

IN USE track claim submissions

clearinghouse

An intermediary processing and transmitting electronic health-care transactions.

IN USE review clearinghouse responses

acknowledgment

A response indicating receipt or initial processing status.

IN USE inspect the claim acknowledgment

front-end rejection

A submission failure before the relevant adjudication stage in the stated workflow.

IN USE correct a front-end rejection

adjudication

Payer review determining how a claim is processed for benefits and payment.

IN USE confirm adjudication status

claim denial

A payer decision not to allow payment as requested after review in this case.

IN USE analyze claim denials

remittance advice

Information explaining payment and adjustments for processed claims.

IN USE review the remittance advice

claim adjustment reason code

A standardized code explaining why a claim payment differs from the billed amount.

IN USE interpret the claim adjustment reason code

remittance advice remark code

A code supplying additional explanation related to an adjustment or processing result.

IN USE review remittance advice remark codes

member identifier

The information identifying the person within the relevant health plan.

IN USE verify the member identifier

payer identifier

The identifier used to direct a transaction to the intended payer.

IN USE validate the payer identifier

Precision check

For this case, rejected means not accepted into adjudication; denied means reviewed and not allowed as requested. Real systems may use labels differently. Verify the acknowledgment and remittance details rather than relying on one word in a dashboard.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

clean claim

A claim meeting the applicable completeness and processing requirements.

IN USE define the clean-claim measure

corrected claim

A submission changing information through the payer's applicable correction process.

IN USE submit a corrected claim

resubmission

Sending a claim again through the appropriate process.

IN USE track the resubmission

appeal

A request to reconsider an adverse determination under the applicable rules.

IN USE evaluate appeal options

timely-filing limit

The applicable deadline for submitting a claim.

IN USE verify the timely-filing limit

appeal deadline

The applicable time limit for requesting review of a decision.

IN USE track the appeal deadline

denial reason

The stated basis for a payer's denial.

IN USE categorize denial reasons

workqueue

A managed list of items awaiting assigned action.

IN USE assign the billing workqueue

accounts receivable

Amounts recorded as due to the organization.

IN USE review accounts receivable

aging bucket

A grouping of outstanding balances by elapsed time.

IN USE monitor aging buckets

patient responsibility

The amount properly assigned to the patient under applicable coverage and billing rules.

IN USE verify patient responsibility

coding integrity

Accurate coding supported by the clinical record and applicable rules.

IN USE preserve coding integrity

Precision check

ERA means electronic remittance advice; CARC, claim adjustment reason code; RARC, remittance advice remark code; and A/R, accounts receivable. These records help explain status and adjustments. An unpaid amount is not automatically patient responsibility, and a correction does not guarantee payment.

Phrases you can actually use

C Notice the language / 1 of 2

SEPARATE THE STAGES

"Twelve submissions were rejected before adjudication; eight claims were denied after review."

REQUEST THE SOURCE

"Show the acknowledgment or remittance record behind this category."

VERIFY BEFORE CORRECTING

"Check the identifier against the appropriate source record."

CHECK THE DEADLINE

"Confirm the applicable filing or appeal deadline for each item."

AVOID AN INFLATED CONCLUSION

"The other eighty claims have not been analyzed here."

NAME THE REJECTION ISSUE

"The member identifier is missing or invalid on these submissions."

ROUTE BY REASON

"Assign follow-up according to the actual response and payer process."

PRESERVE CODING ACCURACY

"Do not change a clinical code solely to obtain payment."

Rejected versus denied

Use the actual transaction stage, since local labels may be inconsistent.

Submitted versus paid

Submission counts do not establish collection results.

Reason plus remark

One code may not contain the full explanation needed for action.

Phrases you can actually use

C Notice the language / 2 of 2

DISTINGUISH NEXT STEPS

"A correction and an appeal are not interchangeable in every workflow."

AVOID A PAYMENT PROMISE

"Resubmission does not establish that the payer will pay."

KEEP RESPONSIBILITY OPEN

"Do not transfer an unresolved amount to the patient without the required review."

PREVENT DOUBLE-COUNTING

"Count a claim consistently as it moves through the workqueues."

CLARIFY THE DENOMINATOR

"The twenty exceptions are out of one hundred submitted claims."

TRACK COMPLETION

"Record the action, owner, date, and resulting status."

EXPLAIN THE ADJUSTMENT

"Read the reason code together with any relevant remark."

CLOSE WITH VISIBILITY

"Show separate rejection and denial totals alongside unresolved follow-up."

Corrected versus original

Follow the payer's actual process instead of creating uncontrolled duplicate submissions.

Deadline

Do not assume one universal filing or appeal period.

Patient balance

Confirm the applicable basis before describing an amount as owed by the patient.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. What percentage of the 20 exceptions are pre-adjudication rejections?
 - A 60%
 - B 12%
 - C 40%
 - D 80%

2. Which dashboard breakdown preserves both stage and denominator?
 - A Twelve rejected claims, eight appeals won, and eighty claims awaiting payment
 - B Twenty rejected submissions out of eighty successfully paid claims
 - C Twelve pre-adjudication rejections and eight reviewed denials out of 100 submissions
 - D Twenty reviewed denials and eighty paid claims out of 100 submissions

3. A rejection cites an invalid member identifier. What should happen first?
 - A Charge the patient immediately because the submission failed.
 - B Verify the identifier and applicable correction process against reliable records.
 - C Replace the diagnosis with a higher-paying code.
 - D Send a generic appeal without checking the stage.

4. Which summary preserves the financial uncertainty?
 - A Correction or appeal may resolve the issue, but payment and patient responsibility remain subject to review.
 - B Every corrected claim must be paid at its full billed amount.
 - C Every denial automatically creates a patient debt.
 - D A submitted appeal is equivalent to an overturned denial.

Answers and explanations: page 87.

One exception total hides two kinds of follow-up

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

member identifier / remittance advice / workqueue / front-end rejections / denial analysis / timely-filing limit / patient responsibility / coding integrity / remittance advice remark code / acknowledgment

- 01 **Leo:** Twenty exceptions out of one hundred submissions. Can I call that a twenty-percent denial rate in the management update?
- 02 **Tessa:** Not from this breakdown. Twelve are ¹ _____ for missing or invalid member identifiers. Only eight reached payer review and were denied.
- 03 **Leo:** Then my appeals-backlog label is wrong too. Where do we check which stage each submission actually reached?
- 04 **Tessa:** Start with its ² _____ and payer response. The dashboard groups work for convenience; it does not establish the correct next action.
- 05 **Leo:** For the twelve identifier errors, could we copy the number from an older paid claim? That would clear them quickly.
- 06 **Tessa:** Verify the current ³ _____ against the appropriate source first. An old claim can be a lead, but it is not permission to guess or reuse unchecked information.
- 07 **Leo:** And for the eight denials, I need more than denied on a list. Can we see why payment was not allowed?
- 08 **Tessa:** Yes. Review the ⁴ _____ and supporting details. A documentation issue and a coverage determination may require different payer-specific follow-up.
- 09 **Leo:** The summary has a broad adjustment reason. Is there another code that explains the missing detail?
- 10 **Tessa:** Check for a ⁵ _____ alongside the adjustment reason and group code. Read them together before assigning correction, reconsideration, or appeal work.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 19.

- 11 **Leo:** Prioritize by deadline as well as category. I do not want older items expiring while we fix the easier ones.
- 12 **Tessa:** We will check each ⁶ _____ and any separate appeal deadline. Sending something again does not necessarily restart the applicable clock.
- 13 **Leo:** A colleague suggested a different diagnosis code on one denial because it might pay. I want that handled properly.
- 14 **Tessa:** That needs qualified review for ⁷ _____, based on the clinical record. Payment preference cannot supply evidence for changing a diagnosis.
- 15 **Leo:** What can I say about the other eighty? The dashboard leaves them outside the exception group, but we have not reviewed them here.
- 16 **Tessa:** Their payment status is unresolved in this meeting. We cannot label them paid or assign ⁸ _____ merely because this view shows no exception.
- 17 **Leo:** Create two lists, then: twelve rejections and eight denials. I need an owner, reason, deadline, and latest status against each claim.
- 18 **Tessa:** I will structure the ⁹ _____ that way and retain claim identity across resubmissions. Several messages about one claim must not become several claims.
- 19 **Leo:** The twelve are sixty percent of exceptions, but twelve percent of all submissions. I will put the denominator next to each percentage.
- 20 **Tessa:** Good. Keep the ¹⁰ _____ focused on the eight adjudicated denials. We can describe the follow-up clearly without promising its payment outcome.

ANSWERS / EXPLANATIONS: PAGE 88

1. front-end rejections | 2. acknowledgment | 3. member identifier | 4. remittance advice | 5. remittance advice remark code | 6. timely-filing limit | 7. coding integrity | 8. patient responsibility | 9. workqueue | 10. denial analysis

The denominator changes the percentage

F Say it

A separate batch contains 200 submitted claims and 30 exceptions. Of the exceptions, 18 were rejected before adjudication and 12 denied after review. No payment status is supplied for the remainder.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

9% / twelve / unspecified / 60%

Analyst: "Rejections are ¹ _____ of all submissions."

Manager: "Rejections are ² _____ of the exceptions."

Analyst: "Reviewed denials total ³ _____."

Manager: "Payment status for the remainder is ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 88

1. 9% | 2. 60% | 3. twelve | 4. unspecified

Rehearse with a fixed script

1. After checking the answers, read the claims discussion. Distinguish twelve front-end rejections from eight post-review denials, with 100 submissions as the total.
2. Switch roles for turns 11-20. Say sixty percent of exceptions and twelve percent of submissions distinctly; do not label the other eighty paid.
3. Complete the separate-batch exchange and check the key. Read eighteen out of 200 as 9%, and eighteen out of thirty as 60%.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Planned discharges are not available beds

A Read the situation

Report usable capacity with a time reference and distinguish physical space from staffed, ready capacity.

A unit has 28 physical beds, of which 24 are staffed and operational today. All 24 operational beds are occupied at the 10 a.m. census. Two discharges are planned, but neither timing nor completion is confirmed. The other four beds are not staffed for use. Flow coordinator Hana and unit manager Paul must correct a message saying two beds are available now. Any new placement depends on confirmed departure, room readiness, appropriate staffing, and the relevant clinical allocation process.

PEOPLE / Hana: Flow coordinator | Paul: Unit manager

ATTACH TIME AND CONDITIONS TO CAPACITY

Capacity figures become misleading when yesterday's position, a forecast, and the current state share the same label. Say available now only when the relevant readiness conditions are confirmed. Explain the next check and owner so a conditional forecast remains useful without becoming an instruction to place a patient.

Briefing checks

Circle one answer in each item. Use the briefing.

1. How many staffed operational beds are unoccupied at 10 a.m.?

- | | |
|---------------|--------------|
| A Zero | B Two |
| C Four | D Six |

2. What is the occupancy rate using staffed operational beds?

- | | |
|--------------------|--------------------|
| A 100% | B About 86% |
| C About 92% | D 50% |

3. What remains unconfirmed?

- | | |
|--|--|
| A The timing and completion of the two planned discharges | B Whether twenty-four beds are occupied at the census |
| C Whether four physical beds are unstaffed | D Whether the census has a stated time |

Answers and explanations: [page 89](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

patient flow

Movement of patients through stages and locations of care.

IN USE coordinate patient flow

bed census

A count of occupied beds at a defined time.

IN USE report the bed census

physical bed capacity

The number of bed spaces physically present.

IN USE distinguish physical bed capacity

staffed capacity

Capacity supported by the required available staffing.

IN USE confirm staffed capacity

operational bed

A bed available for use when applicable operational conditions are met.

IN USE verify operational-bed status

occupancy rate

Occupied capacity divided by a specified capacity denominator.

IN USE state the occupancy-rate denominator

anticipated discharge

An expected departure that has not necessarily occurred.

IN USE track anticipated discharges

confirmed discharge

A departure or discharge status verified under the relevant process.

IN USE distinguish confirmed discharges

discharge barrier

An unresolved condition delaying a planned departure.

IN USE identify discharge barriers

bed turnaround

The process and time needed to prepare a vacated bed for reuse.

IN USE monitor bed turnaround

environmental services

The function providing cleaning and related environmental support.

IN USE coordinate with environmental services

room readiness

Confirmation that a room meets the conditions for its next use.

IN USE verify room readiness

Precision check

Occupancy is 100% on the staffed operational basis: 24 divided by 24. Dividing by 28 gives about 86% physical-bed occupancy, a different measure. Neither calculation makes the four unstaffed beds or two anticipated discharges available now.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

bed allocation

Assignment of a suitable bed through the appropriate process.

IN USE confirm bed allocation

acuity

The level of a patient's clinical complexity or care needs.

IN USE account for patient acuity

skill mix

The combination of staff capabilities available for a workload.

IN USE review the skill mix

staffing coverage

The personnel available to support defined work over a period.

IN USE verify staffing coverage

surge capacity

Additional capacity that can be mobilized under defined conditions.

IN USE assess surge capacity

contingency staffing

Alternative staffing arrangements used under an approved plan.

IN USE confirm contingency staffing

admission demand

The number and nature of patients requiring admission.

IN USE review admission demand

transfer request

A request to move a patient between care locations.

IN USE track transfer requests

boarding

Waiting in a location for an appropriate inpatient placement or next care setting.

IN USE monitor boarding

throughput

The rate at which patients move through a defined process.

IN USE evaluate patient throughput

flow huddle

A brief coordination meeting about current flow and constraints.

IN USE conduct a flow huddle

capacity forecast

An estimate of future usable capacity with stated assumptions.

IN USE qualify the capacity forecast

Precision check

A forecast can inform planning without authorizing placement. A departure may still be followed by cleaning, readiness checks, staffing confirmation, and clinical allocation. Keep each status explicit; no universal staffing ratio or clinical placement rule is supplied by this exercise.

Phrases you can actually use

C Notice the language / 1 of 2

03

TIME-STAMP THE COUNT

"At 10 a.m., all twenty-four staffed operational beds are occupied."

STATE THE DENOMINATOR

"Occupancy is 100% of staffed operational capacity."

KEEP THE FORECAST CONDITIONAL

"Two beds may become available after the required conditions are met."

AVOID AN IMPLIED PLACEMENT

"This forecast does not authorize a patient transfer."

CORRECT THE AVAILABILITY

"The two discharges are planned, not completed."

SEPARATE PHYSICAL SPACE

"Four additional physical beds are not staffed for use today."

NAME THE DEPENDENCY

"Departure, room readiness, staffing, and allocation still need confirmation."

REQUEST A FRESH STATUS

"Please confirm the position before the next flow huddle."

Now versus later

A time reference is essential when capacity changes quickly.

Physical versus staffed

An empty physical space may not be usable capacity.

Discharged versus ready

Departure and readiness are separate operational states.

Phrases you can actually use

C Notice the language / 2 of 2

03

IDENTIFY THE OWNER

"Who will update the bed status after readiness is verified?"

PROTECT THE DISTINCTION

"Vacated does not automatically mean ready for the next patient."

AVOID A FALSE PROMISE

"I cannot report capacity that is not yet confirmed."

LABEL THE ESTIMATE

"This is a capacity forecast, not an available-bed count."

EXPOSE THE BARRIER

"Which unresolved issue affects the expected discharge time?"

CHECK SUITABILITY

"Clinical placement must follow the appropriate assessment process."

COORDINATE THE HANDOFF

"Notify the receiving team when the actual status changes."

CLOSE THE CORRECTION

"Replace available now with anticipated, timing unconfirmed."

Two planned

This describes a forecast, not a completed event.

Occupancy denominator

Always identify whether the measure uses physical, staffed, or another defined capacity.

Placement

An administrative capacity statement does not replace clinical allocation decisions.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which message accurately corrects the original update?

- A** Two beds are available because two discharges appear in the plan.
- B** Four beds are available because physical capacity exceeds occupied beds.
- C** Occupancy is low enough to accept any transfer immediately.
- D** At 10 a.m., no staffed operational beds are free; two discharges are anticipated with timing unconfirmed.

2. What does 24 divided by 28 measure here, and what does it omit?

- A** Physical-bed occupancy; it includes four beds not staffed for use.
- B** Staffed-bed occupancy; it includes two completed discharges.
- C** Available operational capacity; it excludes the four unstaffed spaces.
- D** Post-discharge occupancy; it assumes the two rooms are cleaned.

3. What must follow a confirmed departure before availability is reported?

- A** Automatically mark every adjacent bed ready.
- B** Assume the forecast has supplied clinical approval.
- C** Treat the discharge plan as proof that cleaning already occurred.
- D** Verify the relevant readiness, staffing, and allocation conditions.

4. Which update best supports planning without promising capacity?

- A** No update should be given until every future admission is known.
- B** Two discharges are anticipated; the unit will confirm timing and readiness at the next agreed check.
- C** Two beds are definitely available now, subject to later confirmation.
- D** Physical space alone is sufficient for every placement.

Answers and explanations: [page 89](#).

Planned discharges are not available beds

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

bed allocation / anticipated discharges / flow huddle / capacity forecast / room readiness / occupancy rate / Bed turnaround / staffed capacity / discharge-barrier / bed census

- 01 **Hana:** The transfer desk has been told that two beds are available. Your ten o'clock count shows twenty-four occupied beds. What changed after the count?
- 02 **Paul:** Nothing has been confirmed. The ¹ _____ still shows all twenty-four staffed operational beds occupied. Two departures are planned, but neither has a confirmed completion time.
- 03 **Hana:** Then the message has converted an expectation into present capacity. I will correct it before the receiving team treats it as permission to send someone.
- 04 **Paul:** Please call them ² _____, not available beds. That keeps the information useful for planning while making clear that the patients have not yet left.
- 05 **Hana:** The building has twenty-eight beds. Someone may divide twenty-four by twenty-eight and conclude that four spaces are free for admissions.
- 06 **Paul:** That confuses physical space with ³ _____. The other four beds are not staffed for use today. They are not an immediately usable reserve merely because they appear on the floor plan.
- 07 **Hana:** For today's operational position, twenty-four occupied out of twenty-four staffed beds means full occupancy. We should name the denominator in the update.
- 08 **Paul:** Yes. The ⁴ _____ is one hundred percent on that basis. A physical-bed percentage answers a different question and should not be substituted without explanation.
- 09 **Hana:** Do we know what is holding up the two expected departures? A forecast would be more useful if we understood which conditions remain unresolved.
- 10 **Paul:** We need a current ⁵ _____ check through the responsible team. I will request the status without inventing a departure time or making the clinical decision myself.

The conversation, continued

E Conversations / turns 11-20

03

Continue with the word bank on page 28.

- 11 **Hana:** Even after a patient leaves, the room may not be ready for immediate reuse. The next team needs more than a departure notification.
- 12 **Paul:** Correct. ⁶ _____ must be tracked through the relevant process. Environmental services and the responsible staff need to confirm their parts before the status becomes ready.
- 13 **Hana:** Once a room is ready, who confirms it is suitable for the incoming patient? I do not want a room-status change treated as placement approval.
- 14 **Paul:** The ⁷ _____ process handles that. Capacity language should support the decision, not bypass the appropriate assessment of patient needs, staffing, and the setting.
- 15 **Hana:** I will label the two possible openings as conditional. The transfer desk can plan around the possibility without treating it as a completed availability check.
- 16 **Paul:** That is a ⁸ _____. Include the dependencies and the next update time. A forecast without those limits can quickly become a promise as it moves between teams.
- 17 **Hana:** Who will update the desk when the status changes? I do not want the corrected message to sit unchanged after the room actually becomes ready.
- 18 **Paul:** I will own the unit update and confirm it at the next ⁹ _____. If the position changes sooner, I will notify the desk through the usual channel rather than wait for the meeting.
- 19 **Hana:** I will correct the desk now: no operational bed is free at ten, and the two possible openings depend on discharge and readiness confirmation.
- 20 **Paul:** That matches the facts. We still need ¹⁰ _____ and the other applicable conditions before reporting availability. Keep the current count and the future possibility on separate lines.

ANSWERS / EXPLANATIONS: PAGE 90

1. bed census | 2. anticipated discharges | 3. staffed capacity | 4. occupancy rate | 5. discharge-barrier | 6. Bed turnaround | 7. bed allocation | 8. capacity forecast | 9. flow huddle | 10. room readiness

An empty room still has conditions

F Say it

One patient has left. The room is awaiting cleaning, and staffing for the next placement has not been confirmed. The status system distinguishes vacated from ready.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

ready / vacated / confirmation / pending

Coordinator: "The room is ¹ _____."

Colleague: "Cleaning remains ² _____."

Coordinator: "Staffing still needs ³ _____."

Colleague: "Do not mark the room ⁴ _____ yet."

ANSWERS / EXPLANATIONS: PAGE 90

1. vacated | 2. pending | 3. confirmation | 4. ready

Rehearse with a fixed script

1. Check the cloze key and read the flow huddle. State the ten-o'clock census as twenty-four occupied staffed beds out of twenty-four.
2. Switch roles for the forecast. Keep two anticipated discharges separate from available beds, and retain the four unstaffed physical spaces.
3. Complete and check the vacated-room exchange. Distinguish departure, pending cleaning, unconfirmed staffing, and actual readiness before placement.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Caught in time, but not yet explained

A Read the situation

04

Describe a near miss factually and distinguish a corrective action from evidence that recurrence risk has been reduced.

A patient was assigned to the wrong specialty clinic. A second check caught the error before the appointment, and no clinical harm is reported. The referral record and booking history are available, but the cause has not been established. Quality coordinator Asha and operations lead Ben are preparing a learning review and evidence for an upcoming accreditation discussion. The event must follow the organization's reporting process. A proposed refresher session is not yet supported by a finding that lack of training caused the error.

PEOPLE / Asha: Quality coordinator | Ben: Operations lead

INVESTIGATE THE PROCESS WITHOUT INVENTING BLAME

A factual account separates what happened, how it was detected, the known impact, and what remains uncertain. Thank the person who detected the problem while examining why the earlier steps allowed it. An action plan should address supported contributing factors rather than assume that another reminder will solve everything.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What is established about the event?

A A training failure has been proved.

C The accreditation review has already approved every process.

B The system is guaranteed to prevent recurrence.

D The wrong-specialty booking was caught before the appointment, with no clinical harm reported.

2. Why is a refresher session not yet a complete response?

A Near misses must be excluded from learning reviews.

C The review has not established a training gap or how the action would address the cause.

B A proposed meeting automatically demonstrates effectiveness.

D Training can never be part of a quality response.

3. What should the review use?

A The referral record, booking history, and other relevant verified evidence

C Only the absence of reported clinical harm

B Only the first explanation offered in conversation

D A rewritten record removing the error

Answers and explanations: page 91.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

near miss

A potentially harmful event intercepted or otherwise not resulting in harm.

IN USE report a near miss

safety event

An occurrence relevant to patient safety under the applicable reporting system.

IN USE document a safety event

incident report

A structured record of an event requiring review or follow-up.

IN USE submit an incident report

event chronology

The verified sequence of events and actions.

IN USE reconstruct the event chronology

contributing factor

A condition that helped an event occur or affected its outcome.

IN USE identify contributing factors

root cause analysis

A structured investigation of underlying factors behind an event.

IN USE scope the root cause analysis

systems approach

Examination of how processes and conditions shape performance.

IN USE use a systems approach

human factors

Study of how people interact with tools, tasks, and environments.

IN USE assess human-factors issues

just culture

An approach combining learning, fair accountability, and attention to system conditions.

IN USE support a just culture

psychological safety

A climate in which people can raise concerns without interpersonal fear.

IN USE strengthen psychological safety

detection control

A process intended to identify an error before further consequences.

IN USE evaluate the detection control

preventive control

A process intended to reduce the chance that an error occurs.

IN USE strengthen preventive controls

Precision check

No clinical harm reported is a statement about known impact, not proof that the process is reliable. A successful second check is evidence of detection in this event. The review still needs to understand the earlier failure and whether the control works consistently.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

process map

A representation of the steps and handoffs in a workflow.

IN USE build a process map

failure mode

A way in which a process or component can fail.

IN USE identify a failure mode

risk register

A record of identified risks, responses, and ownership.

IN USE update the risk register

corrective action plan

A documented set of actions addressing an identified problem.

IN USE agree the corrective action plan

action owner

The person accountable for completing a specified action.

IN USE name the action owner

implementation evidence

Records showing that a planned action was carried out.

IN USE collect implementation evidence

effectiveness measure

A defined measure used to judge whether an action worked.

IN USE specify the effectiveness measure

process measure

A measure of whether a defined activity or step occurs as intended.

IN USE track a process measure

outcome measure

A measure of the result the process is intended to influence.

IN USE review the outcome measure

balancing measure

A measure monitoring unintended effects of a change.

IN USE include a balancing measure

accreditation evidence

Material supporting assessment against relevant accreditation requirements.

IN USE organize accreditation evidence

tracer review

Examination following a process or experience across relevant steps and records.

IN USE prepare for a tracer review

Precision check

Root cause analysis is often shortened to RCA. A completed action and an effective action are different claims. Attendance at a refresher session shows participation; a defined follow-up measure is needed to evaluate whether the relevant process actually improved.

Phrases you can actually use

C Notice the language / 1 of 2

STATE THE EVENT

"The patient was booked into the wrong specialty clinic."

LIMIT THE IMPACT CLAIM

"No clinical harm has been reported in the available information."

REQUEST THE SEQUENCE

"Let us reconstruct the booking steps from the records."

VALUE THE DETECTION

"The second check worked in this event; we still need to examine the earlier steps."

REPORT THE INTERCEPTION

"A second check caught the error before the appointment."

KEEP CAUSE OPEN

"The cause has not yet been established."

AVOID PREMATURE BLAME

"We should not label this a training failure without evidence."

CONNECT THE ACTION

"Which verified contributing factor does this proposed action address?"

No harm reported

This preserves the information boundary without minimizing the event.

Cause versus factor

Several interacting conditions may contribute; avoid forcing an unsupported single explanation.

Detected versus prevented

Catching an error after it occurs differs from preventing the initial error.

Phrases you can actually use

C Notice the language / 2 of 2

ASSIGN ACCOUNTABILITY

"Each action needs an owner and completion date."

SEPARATE COMPLETION

"A completed session does not itself demonstrate improved performance."

PREPARE THE EVIDENCE

"Show the event, response, implementation, and follow-up findings."

AVOID A GUARANTEE

"One intercepted event does not prove that recurrence is impossible."

DEFINE EFFECTIVENESS

"What measure would show that the relevant error risk has decreased?"

PRESERVE THE RECORD

"Keep the original booking history and the documented correction."

MONITOR SIDE EFFECTS

"Check whether the change introduces delays or new workarounds."

CLOSE THE REVIEW

"Record supported findings and unresolved questions separately."

Complete versus effective

Task completion needs separate evidence from improved outcomes.

Accreditation

An upcoming review does not justify overstating what the evidence shows.

Learning and accountability

A systems approach does not mean that responsibilities disappear.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which review opening is most precise?

- A** The second check guarantees that this process is safe in every case.
- B** The wrong-specialty booking was intercepted; we will reconstruct the sequence and assess contributing factors.
- C** The scheduler needs training because all booking errors have that cause.
- D** No review is needed because no harm was reported.

2. What does refresher attendance demonstrate?

- A** That the original event was definitely caused by poor training
- B** That every accreditation requirement has been satisfied
- C** That the recorded participants attended, not necessarily that the process improved
- D** That recurrence risk is now zero

3. Which measure most directly tests whether wrong-specialty booking accuracy improved?

- A** Verified wrong-specialty bookings divided by comparable reviewed bookings over the planned period
- B** Staff attending the refresher divided by staff scheduled for it
- C** Completed action-plan tasks divided by tasks assigned
- D** Refresher slides approved divided by slides submitted for review

4. Why include a balancing measure?

- A** To ensure every result can be described as successful
- B** To avoid recording the original error
- C** To detect unintended effects such as added delays or workarounds
- D** To replace the main effectiveness question

Answers and explanations: page 91.

Caught in time, but not yet explained

E Conversations / turns 1-10

04

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

near miss / just culture / effectiveness measure / event chronology / contributing factors / detection control / implementation evidence / action owner / process map / accreditation evidence

- 01 **Ben:** The patient was booked into the wrong specialty, but the second check caught it before the appointment. Can we close the issue as a harmless administrative mistake?
- 02 **Asha:** We should report the ¹ _____ through our process. No clinical harm is reported, but that does not explain the error or establish that the same process will always catch it.
- 03 **Ben:** The referral record and booking history are available. We can identify the sequence without relying only on what people remember after the correction.
- 04 **Asha:** Build the ² _____ from those records and relevant accounts. Keep confirmed facts distinct from assumptions, especially where the timing or handoff is unclear.
- 05 **Ben:** The immediate suggestion is a refresher session for schedulers. That may help, but we have not established that missing knowledge caused this event.
- 06 **Asha:** Identify the ³ _____ first. The interface, referral wording, workflow, or other conditions may need examination. We should not select a cause merely because the proposed action is easy to organize.
- 07 **Ben:** We can thank the person who noticed the mismatch. At the same time, the review should not stop with the fact that the second check worked.
- 08 **Asha:** Exactly. It was a ⁴ _____ in this event. We need to examine the earlier steps and whether the check operates reliably, rather than assuming one successful catch proves universal protection.
- 09 **Ben:** A simple diagram of the referral-to-booking route may help the teams see where information changed or responsibility became unclear.
- 10 **Asha:** Use a ⁵ _____ grounded in actual practice. Include the handoffs and workarounds people really use, not just the ideal sequence written in the procedure.

The conversation, continued

E Conversations / turns 11-20

04

Continue with the word bank on page 37.

- 11 **Ben:** Staff are worried this meeting will start by naming someone to blame. How do we get an honest account of the workarounds without losing accountability?
- 12 **Asha:** A ⁶ _____ supports both learning and fair accountability. Ask what happened and why the process allowed it, while preserving the responsibilities appropriate to each role.
- 13 **Ben:** Once the findings are supported, the action plan should show who will do what. A general instruction to improve accuracy will not be easy to follow up.
- 14 **Asha:** Name an ⁷ _____ and a date for each specific task. Connect each action to the relevant finding so the plan is more than a list of familiar training activities.
- 15 **Ben:** For the accreditation discussion, we can show that an action was completed. But completion alone will not tell us whether it reduced the error risk.
- 16 **Asha:** Separate ⁸ _____ from effectiveness evidence. A revised form or attendance record proves an activity occurred; it does not automatically prove that bookings became more accurate.
- 17 **Ben:** We could review a consistently defined sample of bookings afterward. The measure and review period should be agreed before we decide whether the change worked.
- 18 **Asha:** That provides an ⁹ _____. Also monitor unintended consequences, such as delays or new workarounds, so an apparent improvement in one measure does not hide another problem.
- 19 **Ben:** I will prepare the timeline and mark the open questions. We can show what caught the error without claiming that we have already established its cause.
- 20 **Asha:** Good. The ¹⁰ _____ should reflect the actual state of the work. A clear account of an open investigation is more accurate than a completed-looking story unsupported by the records.

ANSWERS / EXPLANATIONS: PAGE 92

1. near miss | 2. event chronology | 3. contributing factors | 4. detection control | 5. process map | 6. just culture | 7. action owner | 8. implementation evidence | 9. effectiveness measure | 10. accreditation evidence

Attendance is not effectiveness

F Say it

A refresher session has been delivered to all ten scheduled staff. No post-session booking review has yet occurred. The action plan calls for a four-week review of booking accuracy.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

evidence / delivered / review / untested

Lead: "The session has been ¹ _____."

Reviewer: "Attendance supplies implementation ² _____."

Lead: "Effectiveness remains ³ _____."

Reviewer: "Complete the planned four-week ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 92

1. delivered | 2. evidence | 3. untested | 4. review

Rehearse with a fixed script

1. Check the answers and read the booking-error review. State the intercepted wrong-specialty booking without claiming a proven cause or absence of all risk.
2. Switch roles for the action discussion. Contrast refresher attendance with a defined measure of booking accuracy; preserve the original event record.
3. Complete and check the ten-person training exchange. Read the session as delivered and effectiveness as untested until the planned four-week review.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A full patient list for a counting task

A Read the situation

Clarify a data request and apply the appropriate privacy review without turning a limited task into unnecessary disclosure.

05

An operations colleague wants to count last month's missed appointments by clinic. The request asks for every patient's name, date of birth, diagnosis, and full visit notes. A clinic-level count may meet the stated purpose, but neither the appropriate data scope nor the requester's export permission has been confirmed. This is an administrative reporting task, not a provider-to-provider treatment request. Analyst Omar and privacy colleague Nina must confirm the purpose, access, output, and approved sharing route before any extract is sent.

PEOPLE / Omar: Reporting analyst | Nina: Privacy colleague

CLARIFY THE PURPOSE BEFORE CHOOSING THE FIELDS

A privacy question need not sound obstructive. Explain that the requested output should match the task and ask which decision the data must support. Avoid treating internal status, removal of names, or technical access as sufficient evidence that an extract is appropriate. Use the designated privacy process for uncertainty.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What is the stated purpose?

- A** Count missed appointments by clinic for last month
- C** Publish full clinical notes for public use

- B** Review individual treatment decisions between providers
- D** Change every patient's clinical priority

2. What remains unconfirmed?

- A** Appropriate data scope and export permission
- C** Whether the reporting period is last month

- B** Whether a full patient list has been requested
- D** Whether the task concerns missed appointments

3. Which output may meet the stated purpose?

- A** A suitably reviewed clinic-level count
- C** A diagnosis list with names removed and no further review

- B** Full visit notes for every patient by default
- D** Any extract the analyst can technically download

Answers and explanations: [page 93](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

HIPAA

US Health Insurance Portability and Accountability Act; includes relevant privacy and security provisions.

IN USE follow applicable HIPAA requirements

protected health information

Identifiable health information covered by the applicable HIPAA definitions.

IN USE safeguard protected health information

electronic PHI

Protected health information maintained or transmitted electronically.

IN USE protect electronic PHI

covered entity

A health plan, clearinghouse, or qualifying provider covered by HIPAA.

IN USE identify the covered entity

business associate

A party handling PHI for specified functions on a covered entity's behalf.

IN USE assess business-associate responsibilities

business associate agreement

A contract setting relevant duties for a business associate handling PHI.

IN USE verify the business associate agreement

minimum necessary

Reasonable limitation of information to the purpose where the applicable standard applies.

IN USE apply the minimum-necessary standard

permitted use

A use allowed under the applicable legal and organizational basis.

IN USE verify the permitted use

disclosure

Release or access to information outside the entity holding it in the relevant privacy context.

IN USE assess the proposed disclosure

authorization

A specific permission meeting applicable requirements for a defined use or disclosure.

IN USE verify the authorization

role-based access

Access assigned according to work responsibilities.

IN USE enforce role-based access

access entitlement

The permission assigned to a user for defined information or functions.

IN USE review access entitlements

Precision check

The US minimum-necessary standard has exceptions, including disclosures to or requests by health-care providers for treatment. This case concerns administrative reporting, not that treatment exception. Confirm the applicable basis and organizational policy rather than applying one slogan to every information request.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

export permission

Authority to extract information from a system under the relevant controls.

IN USE confirm export permission

purpose limitation

Restricting information handling to a specified appropriate purpose.

IN USE document the purpose limitation

data minimization

Limiting collected or shared data to what is appropriate for the task.

IN USE practice data minimization

aggregate report

A report combining individual records into group-level results.

IN USE review an aggregate report

de-identification

A process meeting applicable criteria for removing or reducing identification risk.

IN USE verify de-identification

re-identification risk

The possibility that information can be linked back to a person.

IN USE assess re-identification risk

small-cell disclosure

Identification risk arising from very small groups in reported data.

IN USE assess small-cell disclosure risk

secure transfer

Sharing through an approved protected method.

IN USE use an approved secure transfer

recipient verification

Confirmation of the intended recipient's identity and authority.

IN USE complete recipient verification

retention schedule

Rules specifying how long records are kept and when they are disposed of.

IN USE follow the retention schedule

audit log

A record of system access or activity used for review.

IN USE review the audit log

privacy incident

An event involving possible inappropriate handling of protected information.

IN USE report a suspected privacy incident

Precision check

PHI means protected health information; ePHI is its electronic form; BAA means business associate agreement. Removing names alone does not establish de-identification. Aggregate outputs can also create identification risk through small groups or linked information, so review the proposed output appropriately.

Phrases you can actually use

C Notice the language / 1 of 2

CLARIFY THE PURPOSE

"Which decision requires this information, and what output is needed?"

CHALLENGE EXCESS FIELDS

"Why would this counting task require full clinical notes?"

SEPARATE CAPABILITY

"Being able to download a file does not establish permission to share it."

APPLY THE RIGHT RULE

"Check the applicable privacy basis and any relevant exceptions."

MATCH THE SCOPE

"A clinic-level count may answer the stated question."

CONFIRM THE ROLE

"Please verify the requester's relevant access and export permissions."

IDENTIFY THE CONTEXT

"This is administrative reporting, not a treatment disclosure request."

AVOID A FALSE SHORTCUT

"Removing names alone does not make the extract de-identified."

Internal

Internal employment does not automatically justify access to every patient record.

Can versus may

Technical capability and authorized use are different.

Minimum necessary

State where the standard applies; do not erase its exceptions.

Phrases you can actually use

C Notice the language / 2 of 2

REVIEW GROUP RISK

"Small counts may still reveal information about individuals."

USE THE APPROVED ROUTE

"Share only through the authorized protected channel."

CONSULT THE OWNER

"Route unresolved privacy questions to the designated reviewer."

DO NOT SEND YET

"Scope and permission are unconfirmed, so the extract remains unsent."

VERIFY THE RECIPIENT

"Confirm identity, authority, and the intended destination."

KEEP A RECORD

"Document the purpose, approved scope, and handling decision."

LIMIT ONWARD USE

"The output should not be repurposed beyond its authorized context."

CLOSE THE REQUEST

"Confirm the approved output and record that the recipient received it appropriately."

Anonymous

Do not use this label merely because a name column was deleted.

Aggregate

Group-level data still need review for identification risk.

Consent versus authorization

Use the precise applicable term rather than assuming all forms of permission are interchangeable.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which proposed output best matches the stated reporting purpose, subject to privacy review?

- A** Every field visible to the analyst because the requester is an employee
- B** Clinic-level monthly missed-appointment counts with appropriate small-group checks
- C** Patient names and diagnoses with visit notes omitted but no purpose review
- D** Full notes with names removed and the result labeled anonymous

2. What does technical download access establish?

- A** That the requester needs all available fields
- B** That privacy review is unnecessary for all exports
- C** That the system permits the action technically, not that the proposed sharing is authorized
- D** That every external destination is approved

3. Which statement about the treatment exception is accurate?

- A** The standard has no exceptions under HIPAA.
- B** Provider treatment disclosures and requests have an exception to minimum necessary; this case is administrative reporting.
- C** Every task performed in a hospital is automatically a treatment disclosure.
- D** The exception means all workforce members may export every record.

4. Why review an aggregate report?

- A** All aggregate reports are automatically prohibited.
- B** Counts always contain full medical notes.
- C** Removing names guarantees that no identification is possible.
- D** Small groups or combined information may still allow identification.

Answers and explanations: [page 93](#).

A full patient list for a counting task

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

05

WORD + PHRASE BANK

audit log / de-identification / export permission / secure transfer / recipient verification / minimum necessary / role-based access / re-identification risk / permitted use / aggregate report

- 01 **Omar:** Operations wants last month's missed appointments by clinic. The request asks for names, dates of birth, diagnoses, and full visit notes. That seems broader than the question.
- 02 **Nina:** Clarify the ¹ _____ and the intended output first. We should understand the decision they need to make before choosing fields from the patient record.
- 03 **Omar:** A count for each clinic may answer the question. I do not see why the report would need individual clinical notes.
- 04 **Nina:** Propose an ² _____ for review. It may meet the purpose without individual details, but we should still assess the output and its disclosure risks.
- 05 **Omar:** The colleague works in operations and can view some records in the system. That does not tell me whether they can receive a broad export.
- 06 **Nina:** Check ³ _____ and the specific permissions. A legitimate role can require some information without authorizing every field or every way of transferring it.
- 07 **Omar:** My reporting account can technically download the full table. I should not treat the availability of an export button as approval for this request.
- 08 **Nina:** Exactly. Confirm ⁴ _____ and the proposed recipient. Technical capability does not settle the purpose, scope, or appropriate destination of the information.
- 09 **Omar:** This is an administrative reporting task. It is not a request between providers to support treatment of an individual patient.
- 10 **Nina:** That distinction matters for ⁵ _____. The US standard has exceptions, including provider treatment disclosures and requests. We should apply the actual rule and policy to this context, not an oversimplified slogan.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 46.

- 11 **Omar:** Would removing the name column be enough? The export would still contain dates of birth, diagnoses, and free-text visit notes.
- 12 **Nina:** That is not established ⁶ _____. Removing one identifier does not prove the extract meets the relevant criteria or eliminates the possibility of linking information back to an individual.
- 13 **Omar:** Even clinic counts could be revealing if a group is very small or the recipient can combine the report with other information.
- 14 **Nina:** Review ⁷ _____ before release. Appropriate aggregation or other controls may be needed, depending on the purpose, context, and applicable review process.
- 15 **Omar:** Once the output is approved, I will verify the recipient and use our designated channel. I will not attach it to an unapproved personal message.
- 16 **Nina:** Use an approved ⁸ _____ method and confirm the destination. Security of the channel matters, but it does not make an excessive or unauthorized disclosure acceptable.
- 17 **Omar:** I will document the purpose, approved fields or counts, and the decision. That will help prevent a later request from silently expanding the same report.
- 18 **Nina:** Preserve the relevant ⁹ _____ and request records under the retention process. The output should remain connected to its authorized purpose rather than becoming an unrestricted reusable patient list.
- 19 **Omar:** I will hold the extract and ask whether clinic-level counts answer the question. Please review the scope and recipient before we send anything.
- 20 **Nina:** Good. Complete ¹⁰ _____ and scope review before sharing. We can support the operational question without assuming that more personal information produces a better report.

ANSWERS / EXPLANATIONS: PAGE 94

1. permitted use | 2. aggregate report | 3. role-based access | 4. export permission | 5. minimum necessary | 6. de-identification | 7. re-identification risk | 8. secure transfer | 9. audit log | 10. recipient verification

An export button is not permission

F Say it

An analyst can download a patient table. The requester needs monthly clinic counts, but the requested export includes diagnoses. Recipient authority and output scope are not yet verified.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

capability / unconfirmed / sharing / purpose

Analyst: "The button shows technical ¹ _____."

Reviewer: "The report's approved scope is ² _____."

Analyst: "Clinic-level counts may meet the stated ³ _____."

Reviewer: "Complete recipient and scope checks before ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 94

1. capability | 2. unconfirmed | 3. purpose | 4. sharing

Rehearse with a fixed script

1. Check the cloze key, then read the data-request discussion. Keep the purpose limited to last month's missed appointments by clinic.
2. Switch roles for the proposed output. Distinguish a reviewed aggregate count, export permission, and a protected sharing route; do not call name removal de-identification.
3. Complete and check the export-button exchange. Keep technical capability separate from authority, with scope and recipient checks unfinished.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Two calls, still no clear answer

A Read the situation

Acknowledge a communication failure, offer an accountable next step, and avoid making a clinical promise.

Patient Leah has called twice to ask when a clinician will discuss a test result. The required identity checks for this call are complete. The call record confirms both messages, but no clinician callback time is confirmed. Leah says this call concerns the missing response, not a current urgent symptom. Administrator Arun can trace the messages, contact the clinical team, and promise his own status update by 3 p.m. He cannot interpret the result, assure Leah that it is normal, or promise a clinical callback at a time the team has not agreed.

PEOPLE / Leah: Patient | Arun: Administrator

06

ACKNOWLEDGE THE EXPERIENCE BEFORE EXPLAINING THE PROCESS

A patient who has already called twice needs recognition and a dependable next step, not another unexplained transfer. State what you can personally do and when you will update them. Keep empathy separate from unsupported reassurance about a result or a clinical decision outside your role.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What can Arun verify?

- A** Both earlier messages appear in the call record.
- B** The result is normal.
- C** A clinician will definitely call at 3 p.m.
- D** Leah has received a clinical explanation.

2. What can Arun promise?

- A** A clinician callback at an unagreed time
- B** That no further clinical follow-up is needed
- C** His own status update by 3 p.m.
- D** A diagnosis during this administrative call

3. What does Leah say about the present call?

- A** She wants the administrator to prescribe treatment.
- B** She has already spoken with the clinician about the result.
- C** It concerns the missing response, not a current urgent symptom.
- D** She is reporting a confirmed medical emergency.

Answers and explanations: page 95.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

patient experience

The interactions a patient has with the health-care system.

IN USE improve patient experience

service recovery

Actions addressing a service failure and its effects on the person affected.

IN USE initiate service recovery

acknowledgment

Recognition of a person's concern or the facts of a situation.

IN USE offer a specific acknowledgment

empathy statement

Language recognizing another person's experience or difficulty.

IN USE use an appropriate empathy statement

communication delay

A gap between needed or expected communication and its occurrence.

IN USE investigate a communication delay

call history

The recorded sequence of telephone contacts and messages.

IN USE review the call history

contact attempt

An effort to reach a person through a communication channel.

IN USE document each contact attempt

callback window

An agreed period during which a return call is expected.

IN USE confirm the callback window

status update

Information about the current state of an unresolved matter.

IN USE provide a status update

resolution commitment

A promise to deliver a specified outcome.

IN USE distinguish an update from a resolution commitment

clinical interpretation

Qualified explanation of medical findings and their meaning.

IN USE route for clinical interpretation

scope of role

The responsibilities and limits of a person's position.

IN USE stay within the scope of role

Precision check

An apology for a communication delay does not require interpreting the test result. A status update promises information about progress, not a clinical conclusion. Explain the difference in ordinary language, particularly when the patient has already waited through earlier unsuccessful contacts.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

warm transfer

A handoff connecting the person with the next team while conveying relevant context.

IN USE arrange a warm transfer

named owner

An identified person accountable for a follow-up action.

IN USE assign a named owner

handoff acceptance

Confirmation that the receiving person or team accepts responsibility for the specified task.

IN USE verify handoff acceptance

contact preference

A person's preferred appropriate method or timing of communication.

IN USE confirm the contact preference

identity verification

Checking that the person is who they claim to be under the relevant process.

IN USE complete identity verification

plain language

Wording intended to be readily understood by the audience.

IN USE use plain language

complaint

An expression of dissatisfaction requiring the applicable response.

IN USE document a complaint

grievance pathway

The designated process for formally reviewing specified patient concerns.

IN USE explain the grievance pathway

de-escalation

Communication intended to reduce tension and support a constructive exchange.

IN USE use respectful de-escalation

expectation setting

Clarifying what will happen, what is uncertain, and who is responsible.

IN USE improve expectation setting

follow-through

Completion of the action or update that was promised.

IN USE demonstrate follow-through

closure confirmation

Checking that the relevant follow-up or resolution has actually occurred.

IN USE seek closure confirmation

Precision check

Administrative staff should use the appropriate clinical route when a clinical concern arises. This case states that Leah is calling about delayed communication, not a current urgent symptom. A promised administrative update must not become a reason to delay appropriate urgent attention in a different situation.

Phrases you can actually use

C Notice the language / 1 of 2

ACKNOWLEDGE THE HISTORY

"I can see both earlier messages in the record."

APOLOGIZE SPECIFICALLY

"I am sorry you have not received a clear update."

SET A CONTROLLED PROMISE

"I will update you myself by 3 p.m."

RESPECT THE ROLE

"I cannot interpret the result, but I can help get your question to the clinical team."

RECOGNIZE THE IMPACT

"I understand why having to call again is frustrating."

STATE YOUR ACTION

"I will trace the messages and contact the clinical team."

SEPARATE THE TIMING

"That is my update time, not yet a confirmed clinician callback time."

AVOID UNSUPPORTED REASSURANCE

"I do not have a basis to tell you the result is normal."

06

I understand

Connect empathy to the actual difficulty rather than using a stock phrase alone.

I will versus they will

Promise your own action; verify another team's commitment before promising theirs.

Normal

Do not infer a clinical result from silence, timing, or administrative status.

Phrases you can actually use

C Notice the language / 2 of 2

KEEP OWNERSHIP

"You should not have to restart the same explanation at each transfer."

EXPLAIN A HANDOFF

"I will pass along the earlier call history and confirm that the team received it."

OFFER THE PROCESS

"I can explain the patient-concern process if you want the delay formally reviewed."

READ BACK THE PLAN

"I will contact the team now and update you by three."

CONFIRM THE CHANNEL

"Let us check the appropriate contact method on your record."

USE PLAIN LANGUAGE

"I am checking who will respond and when, rather than simply moving your message."

UPDATE EVEN WHEN PENDING

"I will contact you at the agreed time even if the clinical response is still unconfirmed."

CONFIRM CLOSURE

"We will check that the promised follow-up actually occurred."

06

Transferred

A transfer is not complete merely because a message was forwarded.

Complaint versus grievance

Follow the organization's applicable definitions and process rather than assigning labels casually.

Update versus resolution

Make clear which outcome is promised and which remains dependent on further action.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which opening best addresses Leah's experience?

- A** No call probably means the result is normal.
- B** You should assume the clinician will call today.
- C** I can see both messages, and I am sorry you have had to call again without a clear update.
- D** The system has a queue, so there is nothing to discuss.

2. Which promise can Arun make without committing another team?

- A** I will close the complaint by three as soon as the clinical team acknowledges receipt.
- B** The result will be ready for explanation by three if no further messages arrive.
- C** I will update you by three, even if the clinician's callback time is still unconfirmed.
- D** The clinical team will explain the result by three because I have forwarded the request.

3. Leah asks whether silence means the result is normal. What should Arun say?

- A** I cannot infer that or interpret the result; I will route the question to the clinical team.
- B** Yes, silence reliably means normal.
- C** The length of the delay determines the diagnosis.
- D** A booking administrator can confirm normality from the call log.

4. What closes the administrative handoff?

- A** Confirm that a clinical team member accepts the follow-up and keep the promised patient update.
- B** Mark the task complete immediately after forwarding a message.
- C** Remove the earlier contacts so only the latest call remains.
- D** Promise a new clinician time without checking it.

Answers and explanations: page 95.

Two calls, still no clear answer

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

clinical interpretation / call history / grievance pathway / status update / named owner / callback window / handoff acceptance / communication delay / follow-through / contact preference

06

- 01 **Leah:** This is my third call about the result. I have answered the identity questions again, but I still do not know who will call me.
- 02 **Arun:** I can see both earlier messages in the ¹ _____. I am sorry you have had to call again. I will trace where those messages went.
- 03 **Leah:** This is about the missing response, not a new urgent symptom. Last time I was told the message had been passed on.
- 04 **Arun:** That ² _____ needs follow-up, not another unexplained transfer. I will contact the clinical team and check who is taking responsibility for your question.
- 05 **Leah:** Does no call mean the result is normal? I keep trying to work out whether the silence tells me anything.
- 06 **Arun:** I cannot give that ³ _____. A missing callback does not tell me what the result means. The qualified clinical team needs to explain it.
- 07 **Leah:** What can you actually promise? I cannot keep waiting beside my phone without knowing whether anyone is dealing with this.
- 08 **Arun:** I will give you a ⁴ _____ myself by three today. I will tell you what the team has confirmed and what is still outstanding.
- 09 **Leah:** So should I keep three o'clock free for the clinician? I do not want to miss the explanation if that is the time.
- 10 **Arun:** We do not have a clinician's ⁵ _____ yet. Three is my update time; I will not describe it as a medical appointment that has not been agreed.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 55.

- 11 **Leah:** Please do not make me explain the first two calls to another person. They are already in the record.
- 12 **Arun:** I will include that history and seek ⁶ _____, so a responsible team member accepts the follow-up. Forwarding your message alone is not enough.
- 13 **Leah:** Use the contact method already on my record. I do not want details sent to a different number during another handover.
- 14 **Arun:** I will confirm that ⁷ _____ through our usual checks. We will use the agreed route rather than assume a new destination is acceptable.
- 15 **Leah:** I also want the delay reviewed. Will raising a complaint distract from getting the result explained?
- 16 **Arun:** Those can proceed separately. I can explain the ⁸ _____ for the communication concern while continuing to seek the clinical response.
- 17 **Leah:** And if you have not got a clinician's time by three? That is usually when the promised update disappears.
- 18 **Arun:** I will still call. That ⁹ _____ is my responsibility. You will hear the actual status and next action, even if the clinical timing remains unresolved.
- 19 **Leah:** All right. Include the earlier messages, find out who is handling it, and contact me by three whether or not the time is confirmed.
- 20 **Arun:** I will remain the ¹⁰ _____ for this administrative follow-up. The clinical team explains the result; I will keep the communication commitment we have agreed.

06

ANSWERS / EXPLANATIONS: PAGE 96

1. call history | 2. communication delay | 3. clinical interpretation | 4. status update | 5. callback window | 6. handoff acceptance | 7. contact preference | 8. grievance pathway | 9. follow-through | 10. named owner

Keep the promised update

F Say it

At 3 p.m., the clinical team has acknowledged the request but has not confirmed a callback time. The administrator promised a status update at 3 p.m.

Complete the four gaps. Use each bank entry once.

06

WORD + PHRASE BANK

update / acknowledged / unconfirmed / interpretation

Administrator: "The clinical team has ¹ _____ the request."

Patient: "Is the callback time confirmed?" Administrator: "It is still ² _____."

Administrator: "This call honors my promised ³ _____."

Patient: "Does receipt establish a result?" Administrator: "No clinical ⁴ _____ follows from receipt alone."

ANSWERS / EXPLANATIONS: PAGE 96

1. acknowledged | 2. unconfirmed | 3. update | 4. interpretation

Rehearse with a fixed script

1. Check the answers and read Leah's call with patient frustration and a calm, specific acknowledgment. Do not interpret the test result from the delay.
2. Switch roles for turns 9-20. Distinguish Arun's three-o'clock update from an unconfirmed clinician callback and seek explicit acceptance of clinical follow-up.
3. Complete and check the three-o'clock exchange. Report acknowledgment accurately while preserving the unconfirmed callback time and lack of clinical interpretation.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Transport pending, but whose next action?

A Read the situation

Hand over unresolved care-transition tasks with verified status, ownership, and a clear escalation condition.

Outgoing coordinator Rachel hands over a discharge coordination case to Imran. The note says transport pending. The transport service has acknowledged the request but has not assigned a vehicle or pickup time. A home-health referral has been received, but its first visit is unconfirmed. Rachel will finish her shift at noon. Imran is to confirm responsibility for both follow-ups and give the care team a status update at 2 p.m. Clinical discharge readiness remains for the qualified team to decide; the administrative handoff does not authorize departure.

PEOPLE / Rachel: Outgoing coordinator | Imran: Incoming coordinator

07

PENDING NEEDS A NOUN, AN OWNER, AND A NEXT CHECK

A short note is useful only when the next person can act on it. Explain what is awaiting confirmation, who holds the next action, and when a response is needed. Repeat the agreed responsibilities so that a shift change does not turn an active request into an unowned task.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What has the transport service confirmed?

- A Clinical discharge readiness
- C A vehicle and pickup time

- B Receipt of the request only
- D Completion of the journey

2. What is the home-health status?

- A The transport provider has accepted clinical responsibility.
- C A first visit is booked at noon.

- B The referral was received; the first visit is unconfirmed.
- D The service has already visited the patient.

3. What does the 2 p.m. commitment mean?

- A The patient is authorized to depart at two.
- C The home-health visit is confirmed for two.

- B A vehicle is guaranteed to arrive at two.
- D Imran will give the care team a status update.

Answers and explanations: [page 97](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

population health

Work addressing health outcomes and needs across a defined group.

IN USE define the population-health cohort

care coordination

Organization of activities and information across people involved in a person's care.

IN USE strengthen care coordination

transition of care

Movement between care settings, providers, or stages.

IN USE support a transition of care

discharge coordination

Organization of practical arrangements related to leaving a care setting.

IN USE document discharge coordination

care plan

The agreed account of needs, goals, and responsibilities for care.

IN USE communicate the care plan

care gap

A difference between needed or planned care and care received.

IN USE identify a care gap

patient panel

A defined group of patients associated with a team or service.

IN USE review the patient panel

risk stratification

Grouping by assessed risk to support appropriate planning.

IN USE review risk-stratification criteria

social needs

Practical social circumstances affecting a person's ability to access or manage care.

IN USE assess reported social needs

transport barrier

A transport-related obstacle to reaching or leaving care.

IN USE resolve a transport barrier

non-emergency medical transport

Transport for medical access when emergency transport is not required, under applicable arrangements.

IN USE coordinate non-emergency medical transport

dispatch confirmation

Verification that the transport provider has made specified dispatch arrangements.

IN USE request dispatch confirmation

Precision check

Request received, service accepted, vehicle assigned, and pickup confirmed describe different steps. Use the provider's actual response. A transport arrangement also does not establish clinical readiness for discharge; that decision remains with the qualified team under the relevant process.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

pickup window

The period during which a pickup is expected.

IN USE verify the pickup window

receiving provider

The professional or organization expected to take the next care role.

IN USE contact the receiving provider

home-health referral

A request for an appropriate service delivered in the home setting.

IN USE track a home-health referral

service acceptance

Confirmation that a receiving service will take the specified referral or task.

IN USE distinguish receipt from service acceptance

first-visit confirmation

Verification of the initial service visit's agreed arrangements.

IN USE obtain first-visit confirmation

medication reconciliation

Qualified comparison and clarification of medication information across care transitions.

IN USE route medication-reconciliation questions

follow-up appointment

A subsequent visit arranged for continued review or care.

IN USE verify the follow-up appointment

community resource

A local service supporting practical or health-related needs.

IN USE verify community-resource availability

handoff checklist

A structured aid for communicating required transition information.

IN USE complete the handoff checklist

closed-loop communication

An exchange including acknowledgment and confirmation of understanding.

IN USE use closed-loop communication

contingency plan

An agreed response if the expected arrangement cannot occur.

IN USE confirm the contingency plan

care-team update

A status communication to the relevant people coordinating care.

IN USE provide a care-team update

Precision check

A handoff should identify current status, open actions, ownership, and timing. SBAR means situation, background, assessment, and recommendation or request. Administrative use of a structured handoff does not authorize an administrator to make a clinical assessment outside their role.

Phrases you can actually use

C Notice the language / 1 of 2

CLARIFY PENDING

"Is transport awaiting acceptance, vehicle assignment, or a pickup time?"

NAME THE NEXT OWNER

"I will take responsibility for the transport follow-up after noon."

SET THE CHECK

"I will update the care team at 2 p.m."

VERIFY SUITABILITY

"The relevant qualified team must confirm the required transport arrangements."

STATE THE CONFIRMED STEP

"The provider acknowledged the request but has not assigned a vehicle."

SEPARATE THE SERVICE

"Home health received the referral; the first visit is not yet confirmed."

AVOID A DEPARTURE PROMISE

"That update time is not authorization for the patient to leave."

CHECK THE NEXT VISIT

"Please confirm the receiving service's first-visit plan."

Pending what

Name the exact step awaiting confirmation.

Received versus accepted

The receiving service may acknowledge a request before agreeing to provide care.

Pickup versus discharge

A transport time and clinical discharge authorization are different facts.

Phrases you can actually use

C Notice the language / 2 of 2

PRESERVE THE DISTINCTION

"Received does not necessarily mean accepted for service."

RECORD THE RESPONSE

"Document the provider's exact status and the time of contact."

READ BACK OWNERSHIP

"You are handing over both follow-ups, and I am accepting them."

AVOID DUPLICATE WORK

"Check the existing request before creating another transport booking."

ESCALATE A BARRIER

"If arrangements remain unresolved, use the agreed escalation route."

CONFIRM THE FALLBACK

"What is the approved contingency if transport cannot be arranged?"

KEEP THE PATIENT INFORMED

"Share the confirmed practical arrangements through the appropriate care-team process."

CLOSE THE HANDOFF

"Record acknowledgment, next actions, and the next update time."

Owner after handoff

Responsibility must be explicitly accepted, not inferred from a copied email.

First visit

A referral alone does not establish when service begins.

Contingency

Use the approved alternative process rather than improvising a care arrangement.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which handoff statement is most actionable?

- A** Transport is booked because the provider replied.
- B** The patient can leave at two because an update is due.
- C** Transport acknowledged the request; no vehicle or time is assigned; Imran owns follow-up and a 2 p.m. update.
- D** Transport pending; someone should check later.

2. Which next check goes beyond a home-health referral receipt?

- A** Whether the sender saved a copy of the original referral email
- B** Whether the referral title contains the expected service name
- C** Whether transport has acknowledged its separate request
- D** Whether the service accepts responsibility and what first-visit arrangements are confirmed

3. Which statement preserves the clinical boundary?

- A** The qualified team determines discharge readiness; administrative coordination supports that decision.
- B** A completed handoff authorizes departure in every case.
- C** A vehicle assignment replaces the clinical discharge process.
- D** An administrator can resolve all medication questions from a transport note.

4. What closes the ownership transfer?

- A** Both coordinators assume the other will call.
- B** Imran acknowledges the tasks and the agreed next actions are documented.
- C** Rachel copies Imran on an email without confirming receipt.
- D** The note is shortened to pending.

Answers and explanations: [page 97](#).

Transport pending, but whose next action?

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

dispatch confirmation / pickup window / contingency plan / handoff checklist / closed-loop communication / care-team update / transport barrier / first-visit confirmation / transition of care / service acceptance

07

- 01 **Rachel:** Before I finish at noon, I need to hand over this discharge coordination case. The note says transport pending, but that wording leaves too much unclear.
- 02 **Imran:** Let us identify the ¹ _____. Has the provider received the request, accepted it, assigned a vehicle, or confirmed a pickup time? Those are different stages.
- 03 **Rachel:** The provider acknowledged receipt. No vehicle or pickup time has been assigned, so we should not describe the journey as booked.
- 04 **Imran:** I will request ² _____ through the existing request. I will check what remains unresolved instead of creating another booking and risking two competing requests.
- 05 **Rachel:** The home-health referral has also been received, but we do not have a confirmed first visit. It belongs on the handoff as a separate open item.
- 06 **Imran:** I will verify ³ _____ and the first-visit plan. The fact that a referral arrived does not tell the care team when the next service will begin.
- 07 **Rachel:** Good. Those arrangements support discharge planning, but neither of us should turn the administrative status into a decision about clinical readiness.
- 08 **Imran:** Agreed. The ⁴ _____ remains coordinated with the qualified team. We will communicate the practical facts without substituting our judgment for the clinical discharge process.
- 09 **Rachel:** I will give you the request references and the last contact times. You should not have to reconstruct everything from a note that simply says pending.
- 10 **Imran:** I will use the ⁵ _____ to capture the open actions. Let us record the provider responses exactly and separate verified information from expected next steps.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 64.

- 11 **Rachel:** You will own both follow-ups after noon. I had planned a care-team update at two, even if the providers had not confirmed the arrangements by then.
- 12 **Imran:** I accept that responsibility and will give the ⁶ _____ at two. I will state what is confirmed, what remains open, and any escalation underway rather than wait silently for a complete answer.
- 13 **Rachel:** If transport is still unresolved, the team needs to know early enough to use the appropriate escalation route. We should not imply that two is a pickup time.
- 14 **Imran:** Correct. Any ⁷ _____ must come from the provider's actual arrangements. The update deadline is our communication commitment, not a transport guarantee.
- 15 **Rachel:** If this transport cannot be arranged, check the approved alternative with the team. Please do not book a different type before suitability and authority are confirmed.
- 16 **Imran:** I will confirm the ⁸ _____ with the relevant team. Suitability and authorization must be established through the proper route before an alternative is reported as available.
- 17 **Rachel:** For home health, please distinguish a general acknowledgment from an actual start date. The receiving team and patient need an accurate expectation.
- 18 **Imran:** I will ask for ⁹ _____ and document the answer. If it is still unknown, I will retain that status and identify who is following up.
- 19 **Rachel:** Before I leave, read back the two follow-ups and the update time. I want to make sure neither task remains assigned to someone whose shift has ended.
- 20 **Imran:** I own transport and home-health follow-up after noon, with a two o'clock status update. That ¹⁰ _____ confirms the tasks, not clinical readiness or departure. I will record the agreed actions and keep the care team informed.

07

ANSWERS / EXPLANATIONS: PAGE 98

1. transport barrier | 2. dispatch confirmation | 3. service acceptance | 4. transition of care | 5. handoff checklist | 6. care-team update | 7. pickup window | 8. contingency plan | 9. first-visit confirmation | 10. closed-loop communication

Acknowledged, not assigned

F Say it

A transport provider confirms receipt of a request but says no vehicle is assigned. The coordinator will check again at 1 p.m.; this is not a pickup promise.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

pickup / check / unassigned / acknowledged

Coordinator: "The request was ¹ _____."

Colleague: "A vehicle remains ² _____."

Coordinator: "One o'clock is the next ³ _____."

Colleague: "Do not present it as a confirmed ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 98

1. acknowledged | 2. unassigned | 3. check | 4. pickup

Rehearse with a fixed script

1. Check the cloze key and read the shift handover. Separate acknowledged transport, no assigned vehicle, and the unconfirmed home-health visit.
2. Switch roles for turns 11-20. Make Imran's acceptance of both follow-ups explicit, with the two-o'clock care-team update unchanged.
3. Complete and check the transport exchange. Read one o'clock as the next check, not pickup, and do not let administrative arrangements authorize discharge.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A shorter wait, or a different reporting window?

A Read the situation

Explain a performance change with comparable definitions and turn a dashboard into a bounded decision request.

A board chart compares last month's median waiting time of 40 minutes across 200 visits with this week's median of 30 minutes across 50 visits. The clinic mix and measurement consistency have not been checked. The headline claims a proven 25% improvement caused by a staffing change. Analyst Jo and executive sponsor Malik must distinguish arithmetic from evidence of sustained improvement or causation. The proposed board request is approval of a four-week validation review, not a claim that staffing changes are already proven effective.

PEOPLE / Jo: Performance analyst | Malik: Executive sponsor

08

EXPLAIN THE COMPARISON BEFORE CELEBRATING THE RESULT

A numerical reduction can be real in the reported values while the operational conclusion remains uncertain. Name the periods, populations, and definitions. Then give decision-makers a specific next step, rather than using caveats either to conceal a weak claim or to avoid making any useful request.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What changed between the compared data sets?

- | | |
|---|--|
| A Only the font used in the chart | B Nothing; both use the same monthly population |
| C The outcome from waiting time to revenue | D The reporting period and visit count, with clinic mix still unchecked |

2. What does 40 to 30 minutes establish arithmetically?

- | | |
|---|---|
| A Proof that every patient's wait fell by ten minutes | B Proof that the staffing change caused the reduction |
| C A confirmed sustained improvement across comparable months | D A ten-minute, 25% reduction between the reported medians |

3. What decision is proposed?

- | | |
|--|--|
| A Approve a four-week validation review | B Declare the staffing change conclusively effective |
| C Approve unlimited additional staffing | D Eliminate measurement checks because the chart improved |

Answers and explanations: page 99.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

executive dashboard

A concise display of measures supporting leadership oversight and decisions.

IN USE review the executive dashboard

key performance indicator

A selected measure of important performance.

IN USE define the key performance indicator

metric definition

The specified calculation, population, and interpretation of a measure.

IN USE document the metric definition

reporting period

The time interval covered by a reported result.

IN USE align reporting periods

measurement window

The interval over which observations are collected for a measure.

IN USE keep the measurement window consistent

numerator

The quantity counted above the line in a defined ratio.

IN USE verify the numerator

denominator

The reference quantity used below the line in a defined ratio.

IN USE state the denominator

median

The middle ordered observation, or mean of the two middle observations for an even count.

IN USE report median waiting time

mean

The total of observed values divided by their number.

IN USE distinguish the mean from the median

percentile

A value indicating a specified position in an ordered distribution.

IN USE examine the ninetieth percentile

case mix

The composition of patients or cases with characteristics relevant to comparison.

IN USE assess case-mix differences

stratification

Separating data into defined groups for analysis.

IN USE use clinic-level stratification

08

Precision check

The reported median fell by ten minutes, or 25% relative to forty. That arithmetic does not show that each patient waited ten minutes less. Nor does it establish a like-for-like trend or prove that a staffing change caused the difference.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

data lineage

The traceable path from source data to a reported result.

IN USE verify data lineage

refresh date

The time a data display was last updated.

IN USE show the refresh date

completeness check

Assessment of whether expected records or fields are present.

IN USE perform a completeness check

like-for-like comparison

A comparison using sufficiently consistent populations, definitions, and periods.

IN USE build a like-for-like comparison

run chart

A time-ordered display of a measure used to examine patterns.

IN USE review the run chart

common-cause variation

Variation inherent in the current process under the relevant statistical model.

IN USE assess common-cause variation

special-cause variation

Variation associated with an identifiable change outside usual process behavior in context.

IN USE investigate potential special-cause variation

causal attribution

A conclusion that a specified factor produced an observed effect.

IN USE qualify causal attribution

relative change

A difference expressed as a proportion of the reference value.

IN USE calculate relative change

absolute change

The numerical difference in the original measurement units.

IN USE report absolute change

data-quality caveat

A stated limitation affecting confidence in a reported result.

IN USE retain a data-quality caveat

decision request

The specific action a decision-making body is asked to authorize.

IN USE state the decision request

Precision check

KPI means key performance indicator. Median, mean, and percentile measures answer different questions. Keep the definition and time basis visible. A run chart can reveal patterns, but a chart alone does not establish causation or remove the need to check data quality.

Phrases you can actually use

C Notice the language / 1 of 2

DESCRIBE THE FIGURES

"The reported medians are forty minutes last month and thirty this week."

LIMIT THE CONCLUSION

"The comparison does not yet establish sustained improvement."

CHECK THE DEFINITION

"Are arrival and service-start times measured consistently?"

REQUEST COMPARABLE DATA

"Rebuild the comparison on aligned periods and definitions."

STATE THE ARITHMETIC

"That is ten minutes lower, or 25% relative to forty."

NAME THE CHANGED BASIS

"The periods and visit counts differ, and clinic mix is unchecked."

PRESERVE THE STATISTIC

"Both figures are medians; do not describe them as every patient's wait."

SEPARATE ASSOCIATION

"The timing of the staffing change does not prove it caused the result."

08

Twenty-five percent

This is relative to the forty-minute reference, not twenty-five percentage points.

Median

A change in a group median does not describe every individual's change.

After versus because of

Sequence in time is not sufficient causal evidence.

Phrases you can actually use

C Notice the language / 2 of 2

SHOW THE LIMITATION

"Place the comparability caveat beside the headline."

LOOK BEYOND ONE POINT

"Review the time pattern rather than declaring a trend from two unmatched values."

BOUND THE AUTHORITY

"This request does not authorize unlimited staffing changes."

KEEP THE FINDING USEFUL

"Report the observed values while marking the conclusion provisional."

VERIFY THE SOURCE

"Trace the measure to its underlying records and calculation."

STATE THE ASK

"Approve a four-week review to validate the comparison."

NAME THE OWNER

"The performance team will own the validation and report-back."

CLOSE WITH EVIDENCE

"Return with comparable results, limitations, and the next decision needed."

08

Comparable

Check definitions, periods, and population composition.

Provisional

Use the qualifier with the conclusion it limits, not only in an appendix.

Approve what

A board update should specify whether it seeks a decision or merely reports information.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which headline accurately describes the observed 40-to-30-minute comparison?

- A** Reported median is ten minutes lower; period, mix, and causal comparability remain under review.
- B** Waiting times fell ten percentage points after the staffing change.
- C** The typical patient is proven to wait 25% less because the newer sample is smaller.
- D** Staffing delivered a sustained 25% improvement, with data validation to follow.

08

2. What is the relative reduction between the reported values?

- A** 75%
- B** 25%
- C** 10%
- D** 33.3%

3. Which validation task is most relevant?

- A** Remove the earlier month because it looks worse.
- B** Assume fifty visits always represent the same mix as two hundred.
- C** Align time periods, definitions, and clinic composition, then review the underlying data.
- D** Replace the median with the smallest wait to strengthen the headline.

4. Which board request matches the brief?

- A** Approve the four-week validation review and require a report-back on comparable results.
- B** End the review because the arithmetic is correct.
- C** Approve any future staffing plan without further evidence.
- D** Treat the chart as a completed causal evaluation.

Answers and explanations: page 99.

A shorter wait, or a different reporting window?

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

metric definition / data lineage / relative change / data-quality caveat / median / decision request / run chart / reporting periods / case mix / Causal attribution

08

- 01 **Malik:** The board chart says waiting time improved by twenty-five percent because of the staffing change. Before I use that headline, are the two figures comparable?
- 02 **Jo:** The ¹ _____ differ. Forty minutes is last month's median across two hundred visits; thirty is this week's median across fifty. We have not checked the clinic mix.
- 03 **Malik:** The arithmetic is still a ten-minute reduction, relative to forty. I want to preserve that fact without suggesting it answers every other question.
- 04 **Jo:** Yes. The ² _____ is twenty-five percent between the reported values. That does not establish a sustained operational improvement or explain why the difference occurred.
- 05 **Malik:** We should also be careful with the word average. Both values are medians, and people may hear that every patient's wait fell by ten minutes.
- 06 **Jo:** Retain ³ _____ in the label. It is a group summary, not a statement about each person's experience. A different summary statistic could tell a different part of the story.
- 07 **Malik:** One clinic may have contributed a larger share of visits this week. If clinics have different waiting patterns, that could affect the combined result.
- 08 **Jo:** Check the ⁴ _____ and use appropriate clinic-level comparisons. We should not assume the smaller weekly sample has the same composition as the previous month's visits.
- 09 **Malik:** Do both extracts start the clock at the same event? Arrival and completed registration would give us different waiting intervals.
- 10 **Jo:** Exactly. Confirm the ⁵ _____ and trace the calculation. A familiar dashboard label does not prove that the underlying start and end events are identical.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 73.

- 11 **Malik:** Can the team reconstruct the data from source records? I want a repeatable calculation rather than a number copied between presentations.
- 12 **Jo:** We will verify ⁶ _____, completeness, and the refresh date. That should show what is included, what is missing, and whether the displayed figures reflect the intended periods.
- 13 **Malik:** The staffing change happened before this week's result, but other factors may have changed too. We should not equate timing with proof.
- 14 **Jo:** Correct. ⁷ _____ needs more than these two values. We can describe the timing without presenting the staffing explanation as established.
- 15 **Malik:** A series over comparable periods would be more informative than selecting one month and one week. We should look at the broader pattern.
- 16 **Jo:** A ⁸ _____ on a consistent basis can help. It should support investigation of the pattern, not serve as a visual shortcut around checking definitions and context.
- 17 **Malik:** For the board, the immediate request is a four-week validation review. It is not a proposal for unlimited staffing changes or a declaration that the intervention succeeded.
- 18 **Jo:** Put that ⁹ _____ up front, with an owner and report-back. The directors should know what they are being asked to authorize before interpreting the chart.
- 19 **Malik:** I will remove the proven-improvement claim and ask for the four-week review. Put the comparability caveat beside the figures, where the board will see it.
- 20 **Jo:** Keep the ¹⁰ _____ beside the finding. We can report the observed values honestly and still offer a practical next step that will make the next decision better informed.

ANSWERS / EXPLANATIONS: PAGE 100

1. reporting periods | 2. relative change | 3. median | 4. case mix | 5. metric definition | 6. data lineage | 7. Causal attribution | 8. run chart | 9. decision request | 10. data-quality caveat

Minutes and percentages are different

F Say it

On an otherwise comparable basis in a separate exercise, median waiting time changes from 50 to 40 minutes. The figures alone do not identify the cause or every patient's experience.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

ten / medians / unestablished / 20%

08

Analyst: "The absolute reduction is ¹ _____ minutes."

Reviewer: "Relative to fifty, the reduction is ² _____."

Analyst: "These are group ³ _____."

Reviewer: "The cause remains ⁴ _____ from these figures alone."

ANSWERS / EXPLANATIONS: PAGE 100

1. ten | 2. 20% | 3. medians | 4. unestablished

Rehearse with a fixed script

1. Check the answers, then read the board-chart review. Distinguish last month's 200 visits from this week's fifty before comparing their medians.
2. Switch roles for the conclusion. State ten minutes or 25% lower, with clinic mix, measurement consistency, and causation still unresolved.
3. Complete and check the separate 50-to-40-minute exchange. Read ten minutes and 20% as different expressions of the change, not individual patient outcomes.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Language assistance belongs in the appointment plan

E More workplace conversations / 1-10

Offer language support respectfully and confirm the practical arrangements.

A patient booking a nonurgent visit says medical discussions in English are difficult. This fictional clinic offers qualified interpreters without charging the patient. The registrar can arrange support but cannot explain clinical consent or substitute for the clinician.

PEOPLE / Huy: Patient | Amira: Patient-access registrar

Use each bank entry once, without changing its form. Answers follow turn 20.

09

WORD + PHRASE BANK

communication need / qualified interpreter / confirmation / remote interpreting / preferred language / confidentiality

- 01 **Huy:** I can arrange the appointment in English, but I find medical explanations difficult. My daughter usually helps me understand.
- 02 **Amira:** Thank you for telling me. What is your ¹ _____ for the visit, including any dialect we should specify?
- 03 **Huy:** Vietnamese. I understand some English, but I worry I will agree to something because I cannot find the right question.
- 04 **Amira:** We can arrange a ² _____ through our service. You will not be charged for that support at this clinic.
- 05 **Huy:** Would my daughter still be allowed to come? I would like her there for support, not to make every decision for me.
- 06 **Amira:** I will record your preference and confirm the visit arrangements. Her supporting role is separate from providing the interpretation.
- 07 **Huy:** Can the interpreter join remotely? It may be hard to find someone who can come at the same time.
- 08 **Amira:** We can request ³ _____. I will confirm availability and the clinic's setup before describing it as booked.
- 09 **Huy:** I would prefer not to explain private matters where everyone in the waiting room can hear.
- 10 **Amira:** I will note that concern and arrange the appropriate setting. Your ⁴ _____ matters during the connection as well as during the consultation.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 76.

- 11 **Huy:** There is a form in the appointment message. Can you explain what the treatment permission section means?
- 12 **Amira:** I can help you obtain language support for the form. The clinical team needs to explain the treatment and answer your questions; I cannot give that explanation for them.
- 13 **Huy:** I sometimes nod when I have heard a sentence, even if I have not understood everything.
- 14 **Amira:** Then please say when you need something repeated. I will note the ⁵ _____ so the team can allow for questions.
- 15 **Huy:** Do I need to call the interpreting company myself?
- 16 **Amira:** No, I will send the request through our process and track the response. You should not have to coordinate the service between several offices.
- 17 **Huy:** Will the appointment confirmation say whether the interpreter is arranged, or only show the doctor's time?
- 18 **Amira:** It will distinguish the visit booking from the interpreter ⁶ _____. If support is still pending, I will explain the next step.
- 19 **Huy:** Thank you. Please use the contact method already recorded for me, and include the language arrangement in the update.
- 20 **Amira:** I will check that contact detail through our usual process and update you with both the appointment and support status.

09

ANSWERS / EXPLANATIONS: PAGE 101

1. preferred language | 2. qualified interpreter | 3. remote interpreting | 4. confidentiality | 5. communication need | 6. confirmation

A requested interpreter is reported as booked

F Say it in a new situation

A clinic has sent an interpreter request but received no acceptance. The appointment itself is booked.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

pending / service / status / update

09

Registrar: The appointment is booked, but interpreting remains
¹ _____.

Colleague: Keep that separate from the visit booking ² _____.

Registrar: I will follow up with the interpreting ³ _____.

Colleague: Tell the patient the actual position in the next ⁴ _____.

ANSWERS / EXPLANATIONS: PAGE 101

1. pending | 2. status | 3. service | 4. update

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

Two similar names, two different patient records

E More workplace conversations / 1-10

Escalate a possible identity mismatch without guessing, merging, or exposing records.

A registrar sees two similar names during check-in. Their dates of birth differ, and an incoming referral may be attached to the wrong record. No clinical information has been disclosed to the waiting patient. The patient-identification team must resolve the mismatch through the approved process.

PEOPLE / Mara: Registration specialist | Felix: Health-information colleague

Use each bank entry once, without changing its form. Answers follow turn 20.

10

WORD + PHRASE BANK

impact assessment / duplicate record / referral provenance / privacy / record merge / patient identifiers

- 01 **Mara:** I have two similar names on screen, and the referral may be attached to the wrong person. I have paused the check-in step.
- 02 **Felix:** Good. Verify the required ¹ _____ through our process, without reading another person's details aloud.
- 03 **Mara:** The dates of birth differ. I have not shown either record to the patient or asked them to choose between the two.
- 04 **Felix:** Keep that boundary. A similar name does not establish a ² _____, and a mismatch does not tell us which record is correct.
- 05 **Mara:** Could I create a third record to get the patient into the queue while we investigate?
- 06 **Felix:** Do not improvise a workaround. Use the identification escalation process so the care team can manage the delay safely without compounding the record problem.
- 07 **Mara:** I will notify the receiving team that check-in is delayed for identification review, not say the patient failed to provide information.
- 08 **Felix:** Exactly. Also preserve the ³ _____ so we can trace how the document was attached.
- 09 **Mara:** I have the source reference and arrival time. Should I move the document to the record with the closer spelling?
- 10 **Felix:** No. Spelling similarity is not enough. The authorized team must establish the right match and apply the correction with traceability.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 79.

- 11 **Mara:** We should check whether anyone has already used the referral information for scheduling or clinical work.
- 12 **Felix:** Yes. That is part of the ⁴ _____. Do not assume there was no effect because you caught it at check-in.
- 13 **Mara:** The patient is asking why this is taking longer. I can explain that we are checking the record without discussing the other person.
- 14 **Felix:** That protects ⁵ _____. Offer a clear update and route any urgent clinical concern promptly to the clinical team.
- 15 **Mara:** If the team confirms that the referral was misfiled, will the original attachment history stay visible?
- 16 **Felix:** The correction must preserve the required history under the authorized process. Do not delete evidence just to make the screen look tidy.
- 17 **Mara:** I will send the discrepancy through the designated channel and keep the check-in status accurate.
- 18 **Felix:** I will confirm the review owner and the next contact. Any ⁶ _____ requires the separate authorized process, not our assumption during this call.
- 19 **Mara:** Please confirm when the match and any affected workflow are resolved. I do not want to restart simply because my message was received.
- 20 **Felix:** Agreed. We will distinguish acknowledgment, identity confirmation, correction, and permission to resume the relevant administrative step.

10

ANSWERS / EXPLANATIONS: PAGE 102

1. patient identifiers | 2. duplicate record | 3. referral provenance | 4. impact assessment | 5. privacy | 6. record merge

Similar names are mistaken for a duplicate

F Say it in a new situation

Two records have similar names but different verified dates of birth. A colleague proposes merging them without identity review.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

delay / person / resolution / merge

10

Registrar: Similar names do not prove these records belong to the same

¹ _____.

Reviewer: Do not perform a record ² _____ on that basis.

Registrar: I will preserve the discrepancy and route it for authorized

³ _____.

Reviewer: Do not disclose the other record while explaining the

⁴ _____.

ANSWERS / EXPLANATIONS: PAGE 102

1. person | 2. merge | 3. resolution | 4. delay

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

A clinic closure needs more than a cancellation message

E More workplace conversations / 1-10

Coordinate administrative disruption while keeping clinical prioritization with the clinical team.

A facilities fault closes one outpatient clinic for the afternoon. Twenty patients are scheduled; none has yet been contacted. Alternative capacity is unconfirmed. Clinical staff must assess any time-sensitive care needs. Administrators are coordinating communication, not independently deciding which care can wait.

PEOPLE / Soren: Clinic operations manager | Priya: Scheduling lead

Use each bank entry once, without changing its form. Answers follow turn 20.

WORD + PHRASE BANK

contact worklist / alternative capacity / status update / handover owner / service-continuity plan / successful notification

11

- 01 **Soren:** Facilities has closed the clinic for this afternoon. We have twenty scheduled patients. How quickly can your team contact them?
- 02 **Priya:** We can begin through the approved routes, but first I need the
1 _____ and the clinical contact for cases that cannot simply be deferred.
- 03 **Soren:** The clinical lead is reviewing that now. Do not assign urgency from appointment labels or move everyone to next week automatically.
- 04 **Priya:** Understood. I will prepare the 2 _____ with the booked visit, approved contact route, and assigned caller.
- 05 **Soren:** Another site may have space, but they have not confirmed staff or suitable appointment types.
- 06 **Priya:** Then we must not advertise that as 3 _____. I will keep it pending until the receiving team confirms what they can accept.
- 07 **Soren:** Some patients may already be traveling. The message needs to say clearly that this site is closed, without implying their care need has disappeared.
- 08 **Priya:** Yes. We will give the verified closure information and the appropriate contact for next steps, with clinical questions passed to the clinical team.
- 09 **Soren:** What will you record when a call goes to voicemail?
- 10 **Priya:** An attempted contact, not a 4 _____. We will follow the approved message and privacy rules.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 82.

- 11 **Soren:** We also need language assistance for some calls. Use the recorded needs rather than assume an English voicemail is enough.
- 12 **Priya:** I will include those arrangements and track which messages still need an accessible follow-up.
- 13 **Soren:** Can you promise everyone a replacement visit tomorrow? That would make the first call easier.
- 14 **Priya:** Not without confirmed slots and clinical review. We can promise a specific
5 _____ from our team and keep it even if rebooking is unfinished.
- 15 **Soren:** Agreed. I will give you a confirmed update time and the route for unresolved cases before calls begin.
- 16 **Priya:** Please also confirm who handles people arriving at the closed site so the phone team and onsite staff give the same information.
- 17 **Soren:** The duty coordinator will handle that under the closure plan. What will your end-of-shift report show?
- 18 **Priya:** Reached, not reached, rebooked, awaiting clinical review, and pending alternatives, with a
6 _____ for every unfinished action.
- 19 **Soren:** That lets us see the remaining work without treating twenty attempted calls as twenty resolved appointments.
- 20 **Priya:** Exactly. We will close each item only when its communication and next-step status meet the actual plan, not when someone has dialed the number.

11

ANSWERS / EXPLANATIONS: PAGE 103

1. service-continuity plan | 2. contact worklist | 3. alternative capacity | 4. successful notification | 5. status update | 6. handover owner

A voicemail is counted as a completed rebooking

F Say it in a new situation

A scheduler leaves a permitted voicemail about a closure. No replacement slot has been offered or accepted.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

rebooked / team / action / handover

Scheduler: The call was attempted; the appointment is not yet ¹ _____.

Lead: Record the voicemail and the required next contact ² _____.

Scheduler: Any question about delaying care goes to the clinical ³ _____.

Lead: Carry the unfinished item into the shift ⁴ _____.

11

ANSWERS / EXPLANATIONS: PAGE 103

1. rebooked | 2. action | 3. team | 4. handover

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

01 Briefing and language checks

Patient Access, Scheduling, and Referrals

A Briefing checks / page 4

1. What information is missing?

D. The requested specialty and reason for the appointment

The referral contains identifiers and contact details but lacks the specialty and appointment reason.

2. What can Elena reliably promise on the supplied facts?

C. A status update by 3 p.m.

Elena controls the update, while booking depends on missing information and unconfirmed availability.

3. Who should determine clinical urgency?

A. The appropriately qualified clinical team through its review process

The brief reserves clinical prioritization for the clinical review process, not administrative guesswork.

D Language checks / page 9

1. Which message matches the referral's actual status?

A. We received the referral and need its specialty and reason; I will update you by 3 p.m.

Receipt and identifiers do not establish acceptance, a held slot, or clinical priority. The supported commitment is a status update while the two missing details are clarified.

2. Which clarification should Marcus request?

C. The intended specialty and clinical reason from the referring team

The missing fields require reliable clarification from the referring team rather than administrative invention.

3. What does the absence of an assigned priority mean?

A. Clinical priority is unconfirmed.

Missing priority information is not evidence of either low or high urgency.

4. At 3 p.m., clarification is still pending. What honors the commitment?

D. Contact the patient with the current status and a clear next update.

The promised action is a status update, so it should occur even when the underlying question remains unresolved.

01 Conversation answers

Patient Access, Scheduling, and Referrals

E Dialogue gaps

Pages 10-11

1. referral completeness. Turn 2. Referral completeness concerns the information required to process the request, and two essential details are missing here.

2. intake queue. Turn 4. The intake queue holds the received request for processing; its presence there does not mean an appointment is arranged.

3. referring clinician. Turn 6. The referring clinician or team can clarify the intended clinical purpose and destination rather than leaving the scheduler to invent them.

4. clinical triage. Turn 8. Clinical triage is the qualified assessment of priority; missing administrative information does not establish a clinical category.

5. callback commitment. Turn 10. The callback commitment promises a status update by a stated time, not a completed appointment booking.

6. booking confirmation. Turn 12. A booking confirmation establishes an arranged appointment; none has been made in this case.

7. scheduling template. Turn 14. The scheduling template defines appointment types and times; an available opening is not automatically suitable for every referral.

8. interpreter request. Turn 16. An interpreter request identifies a language-access need that should be arranged through the appropriate service process.

9. closed-loop referral. Turn 18. A closed-loop referral tracks receipt, action, and feedback so the request does not disappear between teams.

10. escalation pathway. Turn 20. The escalation pathway identifies how unresolved issues reach the appropriate owner without inventing clinical decisions or promises.

F Transfer conversation

Page 12

1. call

Scheduler: "I promised a status call."

The commitment concerns communication at a stated time, not a clinical consultation.

2. pending

Colleague: "The specialist review remains pending."

The supplied facts say review has not been completed.

3. reserved

Scheduler: "No appointment is reserved."

The briefing expressly states that no appointment has been held or booked.

4. confirmation

Colleague: "Make the promised update without implying a booking confirmation."

A status update must not be presented as confirmation of an appointment that does not exist.

02 Briefing and language checks

Revenue Cycle, Coding, and Denials

A Briefing checks / page 13

1. How many exceptions were rejected before adjudication?

C. Twelve

The underlying records identify twelve submission rejections and eight post-review denials.

2. What is known about the other 80 claims?

A. They were not analyzed in this meeting.

The briefing expressly leaves their status unanalyzed, so payment and responsibility cannot be inferred.

3. What must guide the correction or appeal route?

A. The actual response and applicable payer rules

The stage, reason, and payer-specific process determine follow-up; an exception total alone does not.

D Language checks / page 18

1. What percentage of the 20 exceptions are pre-adjudication rejections?

A. 60%

Twelve divided by twenty equals 60%; 12% would use all one hundred submissions as the denominator.

2. Which dashboard breakdown preserves both stage and denominator?

C. Twelve pre-adjudication rejections and eight reviewed denials out of 100 submissions

The records identify twelve rejections before adjudication and eight denials after review. They establish neither appeal outcomes nor the payment status of the other eighty.

3. A rejection cites an invalid member identifier. What should happen first?

B. Verify the identifier and applicable correction process against reliable records.

The stated rejection concerns identifier data, so the first action is accurate verification and route selection.

4. Which summary preserves the financial uncertainty?

A. Correction or appeal may resolve the issue, but payment and patient responsibility remain subject to review.

Taking a follow-up action does not establish its outcome or the correct assignment of financial responsibility.

02 Conversation answers

Revenue Cycle, Coding, and Denials

E Dialogue gaps

Pages 19-20

- 1. front-end rejections.** Turn 2. Front-end rejections occurred before adjudication in this case, unlike the eight claims denied after review.
- 2. acknowledgment.** Turn 4. The acknowledgment records the submission's receipt or processing status and helps establish where it stopped.
- 3. member identifier.** Turn 6. The member identifier identifies the person within the plan; missing or invalid values are the stated rejection cause.
- 4. remittance advice.** Turn 8. The remittance advice explains processed-claim payment and adjustments, helping identify the denial reason and relevant follow-up.
- 5. remittance advice remark code.** Turn 10. A remittance advice remark code adds detail that may be needed to interpret the adjustment or processing result.
- 6. timely-filing limit.** Turn 12. The timely-filing limit governs submission timing and must be verified for the relevant payer and claim context.
- 7. coding integrity.** Turn 14. Coding integrity requires support from the clinical record and applicable rules, not changes selected solely to obtain payment.
- 8. patient responsibility.** Turn 16. Patient responsibility must be established under applicable rules; unresolved or unanalyzed claims do not automatically become patient balances.
- 9. workqueue.** Turn 18. The workqueue organizes follow-up with owners and status, rather than leaving a combined exception total without actions.
- 10. denial analysis.** Turn 20. Denial analysis should use the reviewed-denial group and its actual reasons, not combine every submission exception into one category.

F Transfer conversation

Page 21

- 1. 9%**
Analyst: "Rejections are 9% of all submissions."
Eighteen divided by two hundred equals 9%, using all submissions as the denominator.
- 2. 60%**
Manager: "Rejections are 60% of the exceptions."
Eighteen divided by thirty equals 60%, using only the exception group.
- 3. twelve**
Analyst: "Reviewed denials total twelve."
The briefing identifies twelve claims denied after review, distinct from eighteen rejections.
- 4. unspecified**
Manager: "Payment status for the remainder is unspecified."
The exercise gives no payment information for the remaining claims, so paid status cannot be inferred.

03 Briefing and language checks

Patient Flow, Capacity, and Staffing

A Briefing checks / page 22

1. How many staffed operational beds are unoccupied at 10 a.m.?

A. Zero

All twenty-four staffed operational beds are occupied; planned departures and unstaffed spaces do not create current availability.

2. What is the occupancy rate using staffed operational beds?

A. 100%

Twenty-four occupied beds divided by twenty-four staffed operational beds equals 100%; using all physical beds changes the denominator.

3. What remains unconfirmed?

A. The timing and completion of the two planned discharges

The case supplies the current counts but expressly leaves discharge timing and completion unconfirmed.

D Language checks / page 27

1. Which message accurately corrects the original update?

D. At 10 a.m., no staffed operational beds are free; two discharges are anticipated with timing unconfirmed.

The corrected message preserves the current census and keeps planned departures conditional.

2. What does 24 divided by 28 measure here, and what does it omit?

A. Physical-bed occupancy; it includes four beds not staffed for use.

The denominator 28 counts physical beds, whereas today's staffed operational denominator is 24. The ratio is about 86%, but all 24 operational beds are occupied and neither discharge is confirmed.

3. What must follow a confirmed departure before availability is reported?

D. Verify the relevant readiness, staffing, and allocation conditions.

A departed patient does not by itself establish all conditions needed for the next placement.

4. Which update best supports planning without promising capacity?

B. Two discharges are anticipated; the unit will confirm timing and readiness at the next agreed check.

The useful forecast states its uncertainty and next confirmation step without contradicting the current count.

03 Conversation answers

Patient Flow, Capacity, and Staffing

03

E Dialogue gaps

Pages 28-29

1. bed census. Turn 2. The bed census is the count at a stated time; at 10 a.m. all operational beds were occupied.

2. anticipated discharges. Turn 4. Anticipated discharges are expected departures, not confirmation that beds are empty or ready.

3. staffed capacity. Turn 6. Staffed capacity excludes the four physical beds not staffed for use today.

4. occupancy rate. Turn 8. The occupancy rate is 100% on the stated staffed operational basis: 24 occupied out of 24.

5. discharge-barrier. Turn 10. A discharge-barrier check identifies unresolved conditions affecting departure without assuming that the discharge has occurred.

6. Bed turnaround. Turn 12. Bed turnaround includes preparing a vacated bed for reuse; departure does not automatically establish readiness.

7. bed allocation. Turn 14. Bed allocation assigns a suitable bed through the appropriate process, including relevant clinical and operational requirements.

8. capacity forecast. Turn 16. A capacity forecast estimates future availability under conditions and is distinct from the current available-bed count.

9. flow huddle. Turn 18. The flow huddle is the coordination point for sharing current status, constraints, and responsible follow-up.

10. room readiness. Turn 20. Room readiness confirms the relevant conditions for reuse; it has not yet been established for the anticipated discharges.

F Transfer conversation

Page 30

1. vacated

Coordinator: "The room is vacated."

Departure establishes that the patient has left, not that the room meets all readiness conditions.

2. pending

Colleague: "Cleaning remains pending."

The briefing says the room is awaiting cleaning.

3. confirmation

Coordinator: "Staffing still needs confirmation."

Staffing for the next placement has not been verified.

4. ready

Colleague: "Do not mark the room ready yet."

Cleaning and staffing are unresolved, so the supplied readiness conditions have not been met.

04 Briefing and language checks

Quality, Safety, and Accreditation

04

A Briefing checks / page 31

1. What is established about the event?

D. The wrong-specialty booking was caught before the appointment, with no clinical harm reported.

The brief establishes the booking error and interception, while cause and recurrence protection remain unresolved.

2. Why is a refresher session not yet a complete response?

C. The review has not established a training gap or how the action would address the cause.

The response should follow evidence about contributing factors rather than assume a cause before investigation.

3. What should the review use?

A. The referral record, booking history, and other relevant verified evidence

The available records can help reconstruct the sequence; absence of harm alone does not explain the event.

D Language checks / page 36

1. Which review opening is most precise?

B. The wrong-specialty booking was intercepted; we will reconstruct the sequence and assess contributing factors.

The opening states known facts and a defensible investigation purpose without assuming cause or perfect protection.

2. What does refresher attendance demonstrate?

C. That the recorded participants attended, not necessarily that the process improved

Attendance is implementation evidence; effectiveness requires a relevant measure of the intended improvement.

3. Which measure most directly tests whether wrong-specialty booking accuracy improved?

A. Verified wrong-specialty bookings divided by comparable reviewed bookings over the planned period

A consistently defined booking-error rate measures the intended process result. Attendance and task completion provide implementation evidence, not evidence that the booking process became more accurate.

4. Why include a balancing measure?

C. To detect unintended effects such as added delays or workarounds

A change can improve one aspect while harming another, so monitoring side effects supports a fuller assessment.

04 Conversation answers

Quality, Safety, and Accreditation

E Dialogue gaps

Pages 37-38

1. near miss. Turn 2. The near miss was intercepted before reported clinical harm, but still warrants the applicable reporting and learning process.

2. event chronology. Turn 4. The event chronology reconstructs the verified sequence rather than substituting an assumed explanation.

3. contributing factors. Turn 6. Contributing factors are supported conditions that helped the event occur; lack of training has not yet been established.

4. detection control. Turn 8. The detection control identified an error already made; it is different from preventing the initial wrong booking.

5. process map. Turn 10. The process map shows workflow steps and handoffs, helping locate where the error could enter or escape.

6. just culture. Turn 12. A just culture combines learning about system conditions with fair accountability rather than automatic blame or absence of responsibility.

7. action owner. Turn 14. The action owner is accountable for a defined task; a general aspiration does not assign responsibility.

8. implementation evidence. Turn 16. Implementation evidence shows an action occurred, while effectiveness requires evidence about its result.

9. effectiveness measure. Turn 18. The effectiveness measure tests the intended improvement using a defined, relevant comparison rather than task completion alone.

10. accreditation evidence. Turn 20. Accreditation evidence should accurately show the event, investigation, actions, and follow-up, without overstating completion or effectiveness.

F Transfer conversation

Page 39

1. delivered

Lead: "The session has been delivered."

The briefing confirms completion of the session for the scheduled staff.

2. evidence

Reviewer: "Attendance supplies implementation evidence."

Attendance demonstrates that the activity occurred, not that its intended effect has been achieved.

3. untested

Lead: "Effectiveness remains untested."

No post-session accuracy review has occurred, so the result has not been evaluated.

4. review

Reviewer: "Complete the planned four-week review."

The action plan specifies a follow-up review of booking accuracy as the next evidence-gathering step.

05 Briefing and language checks

HIPAA, Privacy, and Information Governance

A Briefing checks / page 40

1. What is the stated purpose?

A. Count missed appointments by clinic for last month

The brief describes a limited administrative count, not treatment review or publication of identifiable records.

2. What remains unconfirmed?

A. Appropriate data scope and export permission

The purpose and requested fields are supplied, but scope and export authority still require confirmation.

3. Which output may meet the stated purpose?

A. A suitably reviewed clinic-level count

A clinic-level count may answer the reporting question, subject to the applicable privacy and disclosure review.

D Language checks / page 45

1. Which proposed output best matches the stated reporting purpose, subject to privacy review?

B. Clinic-level monthly missed-appointment counts with appropriate small-group checks

The purpose is counting missed appointments by clinic, not reviewing individuals' diagnoses. Aggregation may meet it, but output scope, permissions, and identification risk still need review.

2. What does technical download access establish?

C. That the system permits the action technically, not that the proposed sharing is authorized

System capability does not settle purpose, permission, recipient, or sharing conditions.

3. Which statement about the treatment exception is accurate?

B. Provider treatment disclosures and requests have an exception to minimum necessary; this case is administrative reporting.

The exception is context-specific and does not convert this limited administrative task into a treatment request.

4. Why review an aggregate report?

D. Small groups or combined information may still allow identification.

Aggregation can reduce exposure but does not automatically eliminate identification risk in every output.

05 Conversation answers

HIPAA, Privacy, and Information Governance

E Dialogue gaps

Pages 46-47

1. permitted use. Turn 2. The permitted use must match an appropriate purpose and basis; the limited counting task does not itself justify every requested field.

2. aggregate report. Turn 4. An aggregate report combines records into clinic-level counts that may satisfy the stated purpose with less individual information.

3. role-based access. Turn 6. Role-based access ties permissions to work responsibilities; internal employment does not establish unrestricted access.

4. export permission. Turn 8. Export permission is authority to extract information under the applicable controls, distinct from technical download capability.

5. minimum necessary. Turn 10. Minimum necessary limits information where the standard applies; the provider-treatment exception does not describe this administrative task.

6. de-identification. Turn 12. De-identification requires meeting applicable criteria; deleting names while retaining identifying details is not enough.

7. re-identification risk. Turn 14. Re-identification risk includes linking small groups or combined information to individuals, even in an aggregate report.

8. secure transfer. Turn 16. Secure transfer uses the authorized protected channel, but does not replace the need for an appropriate purpose and recipient.

9. audit log. Turn 18. The audit log records access or activity and supports review of how information was handled.

10. recipient verification. Turn 20. Recipient verification confirms identity and authority before information is shared through the approved route.

F Transfer conversation

Page 48

1. capability

Analyst: "The button shows technical capability."

The system allows an action technically, but that alone does not establish authorization.

2. unconfirmed

Reviewer: "The report's approved scope is unconfirmed."

The briefing says output scope has not yet been verified.

3. purpose

Analyst: "Clinic-level counts may meet the stated purpose."

The operational request concerns monthly counts, which may not require individual diagnoses.

4. sharing

Reviewer: "Complete recipient and scope checks before sharing."

Authority and scope remain unresolved, so the information should not yet be sent.

05

06 Briefing and language checks

Patient Experience and Service Recovery

A Briefing checks / page 49

1. What can Arun verify?

A. Both earlier messages appear in the call record.

The call record confirms the messages, but no clinical interpretation or clinician callback time is supplied.

2. What can Arun promise?

C. His own status update by 3 p.m.

Arun controls his administrative update, not the clinical team's timing or judgment.

3. What does Leah say about the present call?

C. It concerns the missing response, not a current urgent symptom.

The brief explicitly defines this as a communication-delay call and does not provide a current urgent symptom.

D Language checks / page 54

1. Which opening best addresses Leah's experience?

C. I can see both messages, and I am sorry you have had to call again without a clear update.

The opening acknowledges the documented history and frustration without unsupported clinical or timing reassurance.

2. Which promise can Arun make without committing another team?

C. I will update you by three, even if the clinician's callback time is still unconfirmed.

Arun controls his own status call. Forwarding, receipt, and silence do not establish a clinician's agreed time, completed result review, or resolution of the complaint.

3. Leah asks whether silence means the result is normal. What should Arun say?

A. I cannot infer that or interpret the result; I will route the question to the clinical team.

Administrative silence and call records do not establish a clinical interpretation.

4. What closes the administrative handoff?

A. Confirm that a clinical team member accepts the follow-up and keep the promised patient update.

Explicit acceptance assigns the clinical follow-up, while Arun retains his promised patient update. Merely forwarding or receiving a message does not establish responsibility.

06 Conversation answers

Patient Experience and Service Recovery

E Dialogue gaps

Pages 55-56

- 1. call history.** Turn 2. The call history confirms Leah's earlier contacts, so Arun can acknowledge them without asking her to prove she called.
- 2. communication delay.** Turn 4. The communication delay is the unresolved response gap; acknowledging it does not require interpreting the medical result.
- 3. clinical interpretation.** Turn 6. Clinical interpretation requires qualified medical judgment; silence and call records do not establish that a result is normal.
- 4. status update.** Turn 8. The status update is Arun's controlled commitment to report progress, not a promise of a completed clinical discussion.
- 5. callback window.** Turn 10. A callback window must be agreed by the responding team; Arun's update time is not an established clinician appointment.
- 6. handoff acceptance.** Turn 12. Handoff acceptance confirms that the receiving person or team accepts responsibility for the follow-up; sending or receiving a message alone does not establish that.
- 7. contact preference.** Turn 14. The contact preference identifies the agreed communication method and should be checked under the usual verification process.
- 8. grievance pathway.** Turn 16. The grievance pathway provides the appropriate formal route for the concern, separate from clinical interpretation of the result.
- 9. follow-through.** Turn 18. Follow-through means honoring the promised update even when the underlying clinical response remains unresolved.
- 10. named owner.** Turn 20. The named owner is the identified person accountable for this administrative follow-up, not a substitute for the clinician's medical role.

F Transfer conversation

Page 57

1. acknowledged

Administrator: "The clinical team has acknowledged the request."

Receipt has been confirmed, so that limited status can be reported accurately.

2. unconfirmed

Patient: "Is the callback time confirmed?"

Administrator: "It is still unconfirmed."

The briefing explicitly says no clinical callback time has been agreed.

3. update

Administrator: "This call honors my promised update."

The administrator promised a status update at three, not a completed medical discussion.

4. interpretation

Patient: "Does receipt establish a result?"

Administrator: "No clinical interpretation follows from receipt alone."

Acknowledging a message does not establish what the test result means.

07 Briefing and language checks

Population Health and Care Coordination

A Briefing checks / page 58

1. What has the transport service confirmed?

B. Receipt of the request only

Acknowledgment confirms receipt, while vehicle assignment and pickup time remain unconfirmed.

2. What is the home-health status?

B. The referral was received; the first visit is unconfirmed.

The briefing separates receipt of the referral from confirmation of a first visit.

3. What does the 2 p.m. commitment mean?

D. Imran will give the care team a status update.

The stated commitment is communication at a set time, not transport, a visit, or a clinical discharge decision.

D Language checks / page 63

1. Which handoff statement is most actionable?

C. Transport acknowledged the request; no vehicle or time is assigned; Imran owns follow-up and a 2 p.m. update.

The actionable statement contains confirmed status, unresolved steps, ownership, and timing.

2. Which next check goes beyond a home-health referral receipt?

D. Whether the service accepts responsibility and what first-visit arrangements are confirmed

A received referral is not yet service acceptance or a confirmed first visit. The sending record and transport acknowledgment cannot establish the receiving service's commitment.

3. Which statement preserves the clinical boundary?

A. The qualified team determines discharge readiness; administrative coordination supports that decision.

Coordination communicates arrangements but does not transfer clinical decision authority to administrative staff.

4. What closes the ownership transfer?

B. Imran acknowledges the tasks and the agreed next actions are documented.

Explicit acceptance and documented next actions prevent responsibility from disappearing during the shift change.

07 Conversation answers

Population Health and Care Coordination

E Dialogue gaps

Pages 64-65

1. transport barrier. Turn 2. The transport barrier is the unresolved practical arrangement, which must be specified rather than hidden under pending.

2. dispatch confirmation. Turn 4. Dispatch confirmation verifies actual transport arrangements; acknowledgment of a request alone does not provide it.

3. service acceptance. Turn 6. Service acceptance means the receiving service agrees to take the task; simple receipt does not establish that commitment.

4. transition of care. Turn 8. A transition of care involves movement between care settings or stages and requires the appropriate clinical and practical coordination.

5. handoff checklist. Turn 10. The handoff checklist helps preserve required status, references, open actions, and responsibilities during the transfer.

6. care-team update. Turn 12. The care-team update is a promised status communication at 2 p.m., not a guaranteed pickup or clinical discharge time.

7. pickup window. Turn 14. A pickup window must come from confirmed transport arrangements; the administrative update time does not establish it.

8. contingency plan. Turn 16. The contingency plan is the approved response to an unresolved arrangement, not an improvised substitute for assessed transport needs.

9. first-visit confirmation. Turn 18. First-visit confirmation establishes the agreed initial service arrangement, which is not yet supplied by the referral receipt.

10. closed-loop communication. Turn 20. Closed-loop communication includes acknowledgment and confirmation of understanding, making responsibility explicit rather than assumed.

F Transfer conversation

Page 66

1. acknowledged

Coordinator: "The request was acknowledged."

The provider confirmed receipt, which is the only completed transport step supplied.

2. unassigned

Colleague: "A vehicle remains unassigned."

The provider explicitly says no vehicle has been assigned.

3. check

Coordinator: "One o'clock is the next check."

The coordinator promised another status check at one, not transport arrival.

4. pickup

Colleague: "Do not present it as a confirmed pickup."

The briefing excludes a pickup promise and supplies no assigned vehicle.

08 Briefing and language checks

Executive Dashboards and Board Updates

A Briefing checks / page 67

1. What changed between the compared data sets?

D. The reporting period and visit count, with clinic mix still unchecked

One figure is monthly across 200 visits and the other weekly across 50, so comparability needs assessment.

2. What does 40 to 30 minutes establish arithmetically?

D. A ten-minute, 25% reduction between the reported medians

The arithmetic is correct, but it does not establish individual change, causation, or sustained comparable performance.

3. What decision is proposed?

A. Approve a four-week validation review

The supplied request is a bounded review to validate the comparison, not an implementation or causal conclusion.

D Language checks / page 72

1. Which headline accurately describes the observed 40-to-30-minute comparison?

A. Reported median is ten minutes lower; period, mix, and causal comparability remain under review.

Ten minutes divided by forty is a 25% relative difference between medians, not percentage points. Different periods and unchecked clinic mix prevent the headline from establishing a sustained or staffing-caused improvement.

2. What is the relative reduction between the reported values?

B. 25%

Subtract thirty from forty and divide the ten-minute difference by the forty-minute reference value.

3. Which validation task is most relevant?

C. Align time periods, definitions, and clinic composition, then review the underlying data.

The proposed checks address the actual comparability gaps instead of selecting a more favorable number.

4. Which board request matches the brief?

A. Approve the four-week validation review and require a report-back on comparable results.

A bounded validation request addresses uncertainty without claiming proof or expanding the requested authority.

08 Conversation answers

Executive Dashboards and Board Updates

E Dialogue gaps

Pages 73-74

1. reporting periods. Turn 2. The reporting periods differ: one result covers a month and the other a week.

2. relative change. Turn 4. The relative change is ten divided by forty, or 25%, without proving comparability or causation.

3. median. Turn 6. The median summarizes the middle of a group's ordered values; its change does not describe every individual's change.

4. case mix. Turn 8. Case mix describes the composition of visits; changes in clinic contribution may affect the overall comparison.

5. metric definition. Turn 10. The metric definition specifies what interval is measured and must remain consistent across the compared results.

6. data lineage. Turn 12. Data lineage traces the reported measure back to its sources and calculation so the result can be checked.

7. Causal attribution. Turn 14. Causal attribution claims that staffing produced the change; temporal sequence and two unmatched summaries do not establish that.

8. run chart. Turn 16. A run chart displays a measure over time and can support pattern review, while not establishing causation by itself.

9. decision request. Turn 18. The decision request is approval of the bounded validation review, not a claim of proven staffing effectiveness.

10. data-quality caveat. Turn 20. The data-quality caveat makes the comparison limitations visible beside the reported finding rather than hiding them in supporting material.

F Transfer conversation

Page 75

1. ten

Analyst: "The absolute reduction is ten minutes."

Fifty minus forty equals ten minutes, expressed in the original units.

2. 20%

Reviewer: "Relative to fifty, the reduction is 20%."

Divide the ten-minute reduction by the fifty-minute reference to obtain 20%.

3. medians

Analyst: "These are group medians."

The briefing specifies median waiting times, not each individual's change.

4. unestablished

Reviewer: "The cause remains unestablished from these figures alone."

Even a comparable difference does not by itself identify the factor that caused it.

08

09 Conversation answers

Language assistance belongs in the appointment plan

E Extended conversation

Pages 76-77

1. preferred language. Turn 2. Preferred language identifies the language the patient wants to use for communication; basic English conversation does not establish comfort with medical discussions.

2. qualified interpreter. Turn 4. A qualified interpreter provides accurate, impartial spoken communication with appropriate language and specialist skills.

3. remote interpreting. Turn 8. Remote interpreting connects an interpreter by an approved telephone or video service rather than requiring physical attendance.

4. confidentiality. Turn 10. Confidentiality concerns protecting the patient's information; interpretation should not require discussing private matters in an exposed setting.

5. communication need. Turn 14. Communication need records the support required for effective understanding, rather than treating a nod as proof of comprehension.

6. confirmation. Turn 18. Confirmation means the requested support has actually been arranged; a request alone is not that completed status.

F Transfer exchange

Page 78

1. pending

Registrar: The appointment is booked, but interpreting remains pending.

The request has no acceptance, so the interpreting arrangement is not yet confirmed.

2. status

Colleague: Keep that separate from the visit booking status.

The confirmed visit and unconfirmed support have different administrative states.

3. service

Registrar: I will follow up with the interpreting service.

The service needs to respond before the requested support can be treated as arranged.

4. update

Colleague: Tell the patient the actual position in the next update.

An accurate update distinguishes the completed booking from support still being coordinated.

10 Conversation answers

Two similar names, two different patient records

E Extended conversation

Pages 79-80

1. patient identifiers. Turn 2. Patient identifiers are person-specific details used together to match the individual to the correct record, not merely a similar name.

2. duplicate record. Turn 4. A duplicate record is more than one record for the same person; these records have not been shown to belong to one person.

3. referral provenance. Turn 8. Referral provenance identifies where the referral came from and how it entered the record, supporting investigation of the attachment error.

4. impact assessment. Turn 12. Impact assessment examines what actions or disclosures may already have been affected by the mismatch, beyond correcting one attachment.

5. privacy. Turn 14. Privacy requires avoiding unnecessary disclosure of another patient's information while explaining the delay.

6. record merge. Turn 18. A record merge combines records and must follow authorized identity review; it is not a routine fix for similar names.

F Transfer exchange

Page 81

1. person

Registrar: Similar names do not prove these records belong to the same person.

The differing identifiers require review and do not establish one shared identity.

2. merge

Reviewer: Do not perform a record merge on that basis.

Merging combines records and is not justified by name similarity alone.

3. resolution

Registrar: I will preserve the discrepancy and route it for authorized resolution.

The designated process must establish and correct the identity issue with traceability.

4. delay

Reviewer: Do not disclose the other record while explaining the delay.

The patient can receive a useful explanation without learning another person's information.

11 Conversation answers

A clinic closure needs more than a cancellation message

E Extended conversation

Pages 82-83

1. service-continuity plan. Turn 2. The service-continuity plan sets out how essential services and communication are maintained during a disruption.

2. contact worklist. Turn 4. The contact worklist tracks each affected patient's communication status and next action, not just the number of calls attempted.

3. alternative capacity. Turn 6. Alternative capacity means suitable usable service capacity elsewhere; a possible room alone is not confirmation.

4. successful notification. Turn 10. Successful notification means the message has been communicated through the defined process; an unanswered call alone does not establish it.

5. status update. Turn 14. A status update is a communication commitment, distinct from guaranteeing a replacement appointment.

6. handover owner. Turn 18. A handover owner is the named person responsible for unfinished actions after responsibility transfers.

F Transfer exchange

Page 84

1. rebooked

Scheduler: The call was attempted; the appointment is not yet rebooked.

No replacement slot has been offered or accepted, so rebooking is incomplete.

2. action

Lead: Record the voicemail and the required next contact action.

The record should identify what must happen next rather than imply the case is finished.

3. team

Scheduler: Any question about delaying care goes to the clinical team.

Clinical staff, not the scheduler's guess, must assess time-sensitive care questions.

4. handover

Lead: Carry the unfinished item into the shift handover.

The unresolved task needs a clear transfer of responsibility at shift change.

11

Find the move you need

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

01 Patient Access, Scheduling, and Referrals

Acknowledge receipt: We received your referral and are checking the information needed to book.

Avoid an invented reason: I cannot supply a clinical reason from the contact details alone.

Route urgency: Clinical priority needs review by the qualified clinical team.

Full phrase set: pages 7-8 | Dialogue: pages 10-11

02 Revenue Cycle, Coding, and Denials

Separate the stages: Twelve submissions were rejected before adjudication; eight claims were denied after review.

Name the rejection issue: The member identifier is missing or invalid on these submissions.

Preserve coding accuracy: Do not change a clinical code solely to obtain payment.

Full phrase set: pages 16-17 | Dialogue: pages 19-20

03 Patient Flow, Capacity, and Staffing

Time-stamp the count: At 10 a.m., all twenty-four staffed operational beds are occupied.

Separate physical space: Four additional physical beds are not staffed for use today.

Request a fresh status: Please confirm the position before the next flow huddle.

Full phrase set: pages 25-26 | Dialogue: pages 28-29

04 Quality, Safety, and Accreditation

State the event: The patient was booked into the wrong specialty clinic.

Keep cause open: The cause has not yet been established.

Connect the action: Which verified contributing factor does this proposed action address?

Full phrase set: pages 34-35 | Dialogue: pages 37-38

Keep the conversation productive

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

05 HIPAA, Privacy, and Information Governance

Clarify the purpose: Which decision requires this information, and what output is needed?

Confirm the role: Please verify the requester's relevant access and export permissions.

Avoid a false shortcut: Removing names alone does not make the extract de-identified.

Full phrase set: pages 43-44 | Dialogue: pages 46-47

06 Patient Experience and Service Recovery

Acknowledge the history: I can see both earlier messages in the record.

State your action: I will trace the messages and contact the clinical team.

Avoid unsupported reassurance: I do not have a basis to tell you the result is normal.

Full phrase set: pages 52-53 | Dialogue: pages 55-56

07 Population Health and Care Coordination

Clarify pending: Is transport awaiting acceptance, vehicle assignment, or a pickup time?

Separate the service: Home health received the referral; the first visit is not yet confirmed.

Check the next visit: Please confirm the receiving service's first-visit plan.

Full phrase set: pages 61-62 | Dialogue: pages 64-65

08 Executive Dashboards and Board Updates

Describe the figures: The reported medians are forty minutes last month and thirty this week.

Name the changed basis: The periods and visit counts differ, and clinic mix is unchecked.

Separate association: The timing of the staffing change does not prove it caused the result.

Full phrase set: pages 70-71 | Dialogue: pages 73-74

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

absolute change	report absolute change	69
access entitlement	review access entitlements	41
accessibility need	record an accessibility need	6
accounts receivable	review accounts receivable	15
accreditation evidence	organize accreditation evidence	33
acknowledgment	inspect the claim acknowledgment	14
acknowledgment	offer a specific acknowledgment	50
action owner	name the action owner	33
acuity	account for patient acuity	24
adjudication	confirm adjudication status	14
admission demand	review admission demand	24
aggregate report	review an aggregate report	42
aging bucket	monitor aging buckets	15
anticipated discharge	track anticipated discharges	23
appeal	evaluate appeal options	15
appeal deadline	track the appeal deadline	15
appointment slot	confirm an appointment slot	5
audit log	review the audit log	42
authorization	verify the authorization	41
authorization requirement	check authorization requirements	6
balancing measure	include a balancing measure	33
bed allocation	confirm bed allocation	24
bed census	report the bed census	23
bed turnaround	monitor bed turnaround	23

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

boarding monitor boarding	24
booking confirmation issue a booking confirmation	5
business associate assess business-associate responsibilities	41
business associate agreement verify the business associate agreement	41
call history review the call history	50
callback commitment honor a callback commitment	6
callback window confirm the callback window	50
cancellation list check the cancellation list	6
capacity forecast qualify the capacity forecast	24
care coordination strengthen care coordination	59
care gap identify a care gap	59
care plan communicate the care plan	59
care-team update provide a care-team update	60
case mix assess case-mix differences	68
causal attribution qualify causal attribution	69
claim adjustment reason code interpret the claim adjustment reason code	14
claim denial analyze claim denials	14
claim submission track claim submissions	14
clean claim define the clean-claim measure	15
clearinghouse review clearinghouse responses	14
clinical interpretation route for clinical interpretation	50
clinical triage route for clinical triage	5
closed-loop communication use closed-loop communication	60
closed-loop referral support closed-loop referrals	6

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

closure confirmation	seek closure confirmation	51
coding integrity	preserve coding integrity	15
common-cause variation	assess common-cause variation	69
communication delay	investigate a communication delay	50
community resource	verify community-resource availability	60
complaint	document a complaint	51
completeness check	perform a completeness check	69
confirmed discharge	distinguish confirmed discharges	23
contact attempt	document each contact attempt	50
contact preference	confirm the contact preference	51
contingency plan	confirm the contingency plan	60
contingency staffing	confirm contingency staffing	24
contributing factor	identify contributing factors	32
corrected claim	submit a corrected claim	15
corrective action plan	agree the corrective action plan	33
covered entity	identify the covered entity	41
data lineage	verify data lineage	69
data minimization	practice data minimization	42
data-quality caveat	retain a data-quality caveat	69
de-escalation	use respectful de-escalation	51
de-identification	verify de-identification	42
decision request	state the decision request	69
demographic verification	complete demographic verification	6
denial reason	categorize denial reasons	15

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

denominator state the denominator	68
detection control evaluate the detection control	32
discharge barrier identify discharge barriers	23
discharge coordination document discharge coordination	59
disclosure assess the proposed disclosure	41
dispatch confirmation request dispatch confirmation	59
effectiveness measure specify the effectiveness measure	33
electronic PHI protect electronic PHI	41
eligibility verification request eligibility verification	6
empathy statement use an appropriate empathy statement	50
environmental services coordinate with environmental services	23
escalation pathway follow the escalation pathway	6
event chronology reconstruct the event chronology	32
executive dashboard review the executive dashboard	68
expectation setting improve expectation setting	51
export permission confirm export permission	42
failure mode identify a failure mode	33
first-visit confirmation obtain first-visit confirmation	60
flow huddle conduct a flow huddle	24
follow-through demonstrate follow-through	51
follow-up appointment verify the follow-up appointment	60
front-end rejection correct a front-end rejection	14
grievance pathway explain the grievance pathway	51
handoff acceptance verify handoff acceptance	51

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

handoff checklist complete the handoff checklist	60
HIPAA follow applicable HIPAA requirements	41
home-health referral track a home-health referral	60
human factors assess human-factors issues	32
identity verification complete identity verification	51
implementation evidence collect implementation evidence	33
incident report submit an incident report	32
intake queue monitor the intake queue	6
interpreter request arrange an interpreter request	6
just culture support a just culture	32
key performance indicator define the key performance indicator	68
like-for-like comparison build a like-for-like comparison	69
mean distinguish the mean from the median	68
measurement window keep the measurement window consistent	68
median report median waiting time	68
medication reconciliation route medication-reconciliation questions	60
member identifier verify the member identifier	14
metric definition document the metric definition	68
minimum necessary apply the minimum-necessary standard	41
named owner assign a named owner	51
near miss report a near miss	32
network status confirm network status	6
non-emergency medical transport coordinate non-emergency medical transport	59
numerator verify the numerator	68

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

occupancy rate	state the occupancy-rate denominator	23
operational bed	verify operational-bed status	23
outcome measure	review the outcome measure	33
patient access	coordinate patient access	5
patient experience	improve patient experience	50
patient flow	coordinate patient flow	23
patient panel	review the patient panel	59
patient responsibility	verify patient responsibility	15
payer identifier	validate the payer identifier	14
percentile	examine the ninetieth percentile	68
permitted use	verify the permitted use	41
physical bed capacity	distinguish physical bed capacity	23
pickup window	verify the pickup window	60
plain language	use plain language	51
population health	define the population-health cohort	59
preventive control	strengthen preventive controls	32
privacy incident	report a suspected privacy incident	42
process map	build a process map	33
process measure	track a process measure	33
protected health information	safeguard protected health information	41
provisional hold	distinguish a provisional hold from a booking	5
psychological safety	strengthen psychological safety	32
purpose limitation	document the purpose limitation	42
re-identification risk	assess re-identification risk	42

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

reason for referral clarify the reason for referral	5
receiving provider contact the receiving provider	60
receiving service identify the receiving service	5
recipient verification complete recipient verification	42
referral process a referral	5
referral completeness check referral completeness	5
referring clinician contact the referring clinician	5
refresh date show the refresh date	69
relative change calculate relative change	69
remittance advice review the remittance advice	14
remittance advice remark code review remittance advice remark codes	14
reporting period align reporting periods	68
resolution commitment distinguish an update from a resolution commitment	50
resubmission track the resubmission	15
retention schedule follow the retention schedule	42
revenue cycle manage revenue-cycle performance	14
risk register update the risk register	33
risk stratification review risk-stratification criteria	59
role-based access enforce role-based access	41
room readiness verify room readiness	23
root cause analysis scope the root cause analysis	32
run chart review the run chart	69
safety event document a safety event	32
scheduling template review the scheduling template	5

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

scope of role	stay within the scope of role	50
secure transfer	use an approved secure transfer	42
service acceptance	distinguish receipt from service acceptance	60
service recovery	initiate service recovery	50
skill mix	review the skill mix	24
small-cell disclosure	assess small-cell disclosure risk	42
social needs	assess reported social needs	59
special-cause variation	investigate potential special-cause variation	69
specialty	confirm the requested specialty	5
staffed capacity	confirm staffed capacity	23
staffing coverage	verify staffing coverage	24
status update	provide a status update	50
stratification	use clinic-level stratification	68
surge capacity	assess surge capacity	24
systems approach	use a systems approach	32
throughput	evaluate patient throughput	24
timely-filing limit	verify the timely-filing limit	15
tracer review	prepare for a tracer review	33
transfer request	track transfer requests	24
transition of care	support a transition of care	59
transport barrier	resolve a transport barrier	59
waitlist	maintain the waitlist	6
warm transfer	arrange a warm transfer	51
workqueue	assign the billing workqueue	15

Original cases. Clear boundaries.

These are original fictional teaching conversations, not transcripts of real workers. Names, figures, organizations, and incidents are invented. Sources support terminology and communication aims, not the fictional results or an endorsement of this book.

[1] US Department of Health and Human Services. Minimum Necessary Requirement.

[Open the original source](#)

Background for the US privacy example, including the standard's exceptions. The exercise is not a universal rule for every disclosure. Checked 10 October 2026.

[2] Centers for Medicare & Medicaid Services. Health Care Payment and Remittance Advice.

[Open the original source](#)

Background on remittance information and adjustment explanations. Fictional claim counts do not prescribe payer-specific correction or appeal steps. Checked 10 October 2026.

[3] Agency for Healthcare Research and Quality. What Is Patient Experience?

[Open the original source](#)

Background on patient interactions, timely information, and communication. The scheduling and service-recovery dialogues are original. Checked 10 October 2026.

[4] AHRQ Patient Safety Network. Communication During Transitions of Care.

[Open the original source](#)

Background on clear handoffs and coordination. The administrative cases do not replace clinical transition protocols. Checked 10 October 2026.

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